GULF COAST VASCULAR SURGERY ASSOCIATES

2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321

PATIENT MEDICAL CHART — CONFIDENTIAL — FOR AUTHORIZED USE ONLY

Consolidated record compiled September 22, 2026 for prior authorization (Florida Blue PPO). Includes outside records received from Westchase Family Medicine, Bayshore Dermatology Group, Tampa Bay Physical Therapy & Rehabilitation and Carrollwood Urgent Care under signed release. Encounters are in chronological order.

1. PATIENT DEMOGRAPHICS

Patient Name	Elena Ruiz
Date of Birth	March 4, 1968
MRN	GCV-012877
Age	58 years
Sex	Female
Address	3318 W Azeele St, Tampa, FL 33609
Phone	(813) 555-0172
Occupation	Elementary school teacher (4th grade), Hillsborough County Public Schools
Primary Care Physician	Dr. Priya Raman, MD
Emergency Contact	Sofia Ruiz (daughter), (813) 555-0119

2. INSURANCE & COVERAGE

Primary Insurance

Payer	Florida Blue (Blue Cross and Blue Shield of Florida)
Plan Type	PPO — BlueOptions
Member ID	FLB771204936
Group Number	FL-55120-OPT
Coverage Effective	January 1, 2023
Prior Auth Required	Yes — Vein procedures
Auth Contact	1-800-555-0199

Secondary Insurance

None on file.

3. ACTIVE PROBLEM LIST

ICD-10	Diagnosis	Onset	Status
I83.812	Varicose veins of left lower extremity with pain	2023	Active
I87.2	Venous insufficiency (chronic) (peripheral), left	2023	Active
I10	Essential hypertension	2018	Active
E03.9	Hypothyroidism, unspecified	2016	Active

ICD-10	Diagnosis	Onset	Status
E78.5	Hyperlipidemia, unspecified	2024	Active
E55.9	Vitamin D insufficiency	2024	Active
Z86.718	Personal history of venous thrombosis (right posterior tibial vein DVT 06/2025, provoked, treated)	2025	Resolved
Z85.828	Personal history of basal cell carcinoma, right shoulder (ED&C 10/2025)	2025	Resolved
L24.9	Irritant contact dermatitis, hands	2024	Controlled
M54.50	Low back pain (lumbar DDD), resolved with PT	2026	Resolved
M22.2X1	Patellofemoral pain, right knee, resolved with PT	2024	Resolved

4. CURRENT MEDICATIONS

As of September 22, 2026.

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

5. MEDICATION HISTORY

Medication	Dose	Frequency	Start	Stop	Indication
Levothyroxine	50 mcg	Daily	02/01/2016	10/15/2024	Hypothyroidism
Losartan	50 mg	Daily	04/01/2018	01/23/2024	Hypertension
Ibuprofen	400 mg	PRN	10/24/2023	06/10/2025	Left leg aching
Amoxicillin-clavulanate	875/125 mg	BID x 7 days	01/09/2024	01/16/2024	Acute bacterial sinusitis
Losartan	100 mg	Daily	01/23/2024	—	Hypertension
Atorvastatin	20 mg	Nightly	05/21/2024	01/20/2026	Hyperlipidemia
Cephalexin	500 mg	QID x 7 days	08/06/2024	08/13/2024	Cellulitis, right forearm
Levothyroxine	75 mcg	Daily	10/15/2024	—	Hypothyroidism
Cholecalciferol (Vitamin D3)	2000 IU	Daily	10/15/2024	—	Vitamin D insufficiency

Medication	Dose	Frequency	Start	Stop	Indication
Triamcinolone 0.1% cream	Topical	BID PRN	11/12/2024	02/11/2025	Hand dermatitis
Benzonatate	100 mg	TID PRN x 7 days	01/14/2025	01/21/2025	Cough, acute bronchitis
Acetaminophen	500 mg	PRN	06/10/2025	—	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Apixaban	10 mg	BID x 7 days (loading)	06/10/2025	06/17/2025	Acute DVT, right calf
Apixaban	5 mg	BID	06/17/2025	09/16/2025	Acute DVT, right calf (3-month course)
Ondansetron ODT	4 mg	Q8H PRN	12/09/2025	12/12/2025	Nausea, viral gastroenteritis
Atorvastatin	40 mg	Nightly	01/20/2026	—	Hyperlipidemia
Cyclobenzaprine	5 mg	QHS PRN	02/17/2026	03/17/2026	Lumbar muscle spasm
Naproxen	500 mg	BID x 7 days	07/07/2026	07/14/2026	Right ankle sprain

6. ALLERGIES & ADVERSE REACTIONS

Cephalexin — rash (08/2024, mild, non-urticarial).

7. ENCOUNTER NOTES — ALL SOURCES, CHRONOLOGICAL

#	Date	Source	Type
1	10/10/2023	Primary Care	Annual Physical / Preventive Visit
2	10/24/2023	Vascular Surgery	New Patient Consult
3	12/05/2023	Vascular Surgery	Follow-Up (duplex review)
4	01/09/2024	Urgent Care	Urgent Care Visit
5	01/23/2024	Primary Care	Chronic Care Follow-Up (hypertension)
6	02/13/2024	Vascular Surgery	Follow-Up (conservative management)
7	03/19/2024	Physical Therapy	Physical Therapy Initial Evaluation
8	04/02/2024	Physical Therapy	Physical Therapy Treatment (visit 3 of 12)
9	04/16/2024	Physical Therapy	Physical Therapy Treatment (visit 6 of 12)
10	05/21/2024	Primary Care	Chronic Care Follow-Up (hypertension, lipids)
11	07/16/2024	Dermatology	New Patient — Full Skin Examination
12	08/06/2024	Urgent Care	Urgent Care Visit
13	09/17/2024	Vascular Surgery	Follow-Up (conservative management)
14	10/15/2024	Primary Care	Annual Physical / Preventive Visit
15	11/12/2024	Dermatology	Follow-Up
16	12/17/2024	Vascular Surgery	Follow-Up (3-month compression check)
17	01/14/2025	Urgent Care	Urgent Care Visit
18	02/11/2025	Primary Care	Chronic Care Follow-Up
19	03/18/2025	Vascular Surgery	Follow-Up (conservative management)
20	04/22/2025	Physical Therapy	Physical Therapy Initial Evaluation
21	05/06/2025	Physical Therapy	Physical Therapy Treatment (visit 4 of 12)
22	06/10/2025	Urgent Care	Urgent Care Visit
23	06/17/2025	Primary Care	Acute Follow-Up (DVT)
24	07/15/2025	Vascular Surgery	Follow-Up (conservative management; interval DVT)
25	08/19/2025	Dermatology	Annual Full Skin Examination
26	09/16/2025	Primary Care	Chronic Care Follow-Up (DVT completion)
27	10/21/2025	Primary Care	Annual Physical / Preventive Visit
28	11/18/2025	Vascular Surgery	Follow-Up (pre-procedure planning)
29	12/09/2025	Urgent Care	Urgent Care Visit
30	01/20/2026	Primary Care	Chronic Care Follow-Up (lipids)
31	02/17/2026	Physical Therapy	Physical Therapy Initial Evaluation
32	03/03/2026	Physical Therapy	Physical Therapy Treatment (visit 4 of 12)
33	03/17/2026	Physical Therapy	Physical Therapy Treatment (visit 8 of 12) — Discharge
34	04/14/2026	Vascular Surgery	Follow-Up (pre-procedure planning)
35	05/12/2026	Dermatology	Follow-Up (BCC site) / Skin Check
36	06/09/2026	Primary Care	Chronic Care Follow-Up (lipids)

#	Date	Source	Type
37	07/07/2026	Urgent Care	Urgent Care Visit
38	08/11/2026	Vascular Surgery	Follow-Up (procedure scheduling)
39	09/08/2026	Primary Care	Pre-Procedure Evaluation
40	09/22/2026	Vascular Surgery	Follow-Up (duplex review; prior authorization)

Visit Date: October 10, 2023 | Annual Physical / Preventive Visit

Encounter	#1 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Annual Physical / Preventive Visit
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 55
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Annual physical. Also reports left leg aching and tiredness.

History of Present Illness:

55-year-old female teacher presents for her annual preventive visit. Chronic problems: essential hypertension on losartan 50 mg, controlled on home readings 120s–130s/70s–80s; primary hypothyroidism on levothyroxine 50 mcg, last TSH 2.8 one year ago. She eats a mostly Mediterranean diet, does not smoke, drinks 2–3 glasses of wine per week, and walks most evenings.

New complaint: for about 6 months the left leg has felt heavy and achy by the end of the school day. She stands and walks in the classroom for most of a 7-hour day. She has noticed a few visible veins on the inside of the left calf and some ankle puffiness in the evenings that is gone by morning. No pain at rest, no cramping yet, no skin changes. Family history: mother had varicose veins stripped in her 50s. Patient has had two pregnancies.

Health maintenance: mammogram due; colonoscopy 2021 (normal, repeat 10 years); Pap/HPV 2022 normal; Tdap 2019; influenza vaccine today.

Vital Signs:

Blood Pressure	135/82 mmHg, seated, left arm
Heart Rate	66 bpm, regular
Temperature	98.4 °F oral
Respiratory Rate	17 /min
SpO2	99% on room air
Weight	172 lb
Height	64 in
BMI	29.5 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.

Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Reports left leg aching and tiredness by end of the day, several months. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Essential hypertension, controlled. Continue losartan 50 mg daily. CMP and lipid panel today.

2. Hypothyroidism. Continue levothyroxine 50 mcg. TSH today.

3. Left lower extremity heaviness and aching with visible medial calf veins and evening ankle edema in a patient who stands all day; clinical picture consistent with early superficial venous insufficiency. Referral placed to Gulf Coast Vascular Surgery Associates (Dr. Vasquez) for evaluation. Advised elevation after work and walking.

4. Preventive: influenza vaccine administered today. Screening mammogram ordered. Return 1 year or sooner as needed.

Orders:

• CMP, lipid panel, TSH, CBC — fasting

• Referral: Vascular Surgery (left leg varicose veins / venous insufficiency)

• Screening mammogram, bilateral

• Influenza vaccine, administered

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	50 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99396	Preventive visit, established, 40–64
I10	Essential hypertension
E03.9	Hypothyroidism
I83.92	Asymptomatic varicose veins of left lower extremity — referred for evaluation

Electronically signed: Dr. Priya Raman, MD — October 10, 2023 10:41

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: October 24, 2023 | New Patient Consult

Encounter	#2 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	New Patient Consult
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 55
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Left leg heaviness, aching and evening swelling; visible veins medial calf. Referred by Dr. Raman.

History of Present Illness:

55-year-old female 4th-grade teacher referred by her PCP for approximately 6 months of progressive left lower extremity heaviness and aching that builds through the school day and is worst by late afternoon. Ankle swelling by evening, resolved by morning. Relieved by elevation and by sitting with the leg up for 20 minutes. Occasional itching over the medial calf. No night cramps yet. No ulcer, no bleeding, no redness along a vein. The right leg is asymptomatic.

No personal history of deep vein thrombosis, pulmonary embolism, or superficial thrombophlebitis. No prior venous procedures, no prior sclerotherapy. No known clotting disorder. Two uncomplicated pregnancies. Does not smoke. Not on hormonal therapy.

Risk factors: prolonged occupational standing, positive family history (mother), BMI 29. She has not tried compression stockings. She takes ibuprofen about twice a week for the aching.

Functional impact at this time: she finishes the school day but 'collapses on the couch' and has reduced her evening walks from 5 nights to 2–3 nights per week.

Vital Signs:

Blood Pressure	138/86 mmHg, seated, left arm
Heart Rate	74 bpm, regular
Temperature	97.7 °F oral
Respiratory Rate	15 /min
SpO2	98% on room air
Weight	173 lb
Height	64 in
BMI	29.7 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.

Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Scattered reticular veins and a few small non-tortuous varicosities over the medial calf in the GSV distribution. Trace ankle edema at the end of the day per patient; none at rest this morning. No pigmentation, lipodermatosclerosis, eczema or ulceration. Calf soft, non-tender, Homans negative.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic left lower extremity superficial venous insufficiency, suspected great saphenous vein distribution. CEAP clinical class C1–C2 on today's exam (reticular veins and small varicosities), Es, As, Pr pending duplex.

2. Plan: lower extremity venous reflux duplex ultrasound, left, to assess the deep system, the saphenofemoral junction and the GSV for reflux. Conservative measures reviewed: leg elevation, calf exercise, weight management, ibuprofen PRN. Compression stockings discussed in principle; patient would like to see the ultrasound result first. Return after the duplex to review and set a plan.

Orders:

• Lower extremity venous reflux duplex, left (CPT 93970/93971)

• Return visit after duplex

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	50 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching

Changes since last encounter:

• STARTED Ibuprofen 400 mg PRN — Left leg aching (10/24/2023)

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99204	New patient visit, moderate complexity
I83.92	Varicose veins, left lower extremity
R60.0	Localized edema

Electronically signed: Dr. Elena Vasquez, MD, FACS — October 24, 2023 15:19

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: December 5, 2023 | Follow-Up (duplex review)

Encounter	#3 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (duplex review)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 55
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Review of left leg venous duplex; ongoing heaviness.

History of Present Illness:

Returns to review the 11/14/2023 left lower extremity venous duplex. Symptoms unchanged since October: heaviness and aching by mid-afternoon on teaching days, evening ankle swelling, better on weekends and when she elevates. Ibuprofen 400 mg two to three times per week. No new symptoms. No cramping, no skin change, no ulcer.

Duplex (reviewed with patient): deep venous system patent and compressible, no DVT. Saphenofemoral junction with brief reflux of 0.4 seconds on Valsalva, which is below the 0.5 second threshold used in this practice; GSV diameter 3.9 mm at the junction and 3.2 mm at mid-thigh. Small saphenous vein competent. No incompetent perforators.

Vital Signs:

Blood Pressure	128/76 mmHg, seated, left arm
Heart Rate	81 bpm, regular
Temperature	98.3 °F oral
Respiratory Rate	17 /min
SpO2	98% on room air
Weight	176 lb
Height	64 in
BMI	30.2 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.

Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Medial calf: cluster of three small varicosities in the GSV distribution, the largest measuring approximately 2 mm in diameter, with surrounding reticular veins. Trace pitting edema at the medial malleolus at 2:40 pm. No pigmentation, no eczema, no lipodermatosclerosis, no ulcer. Calf soft and non-tender.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Left lower extremity symptoms with early, sub-threshold GSV reflux on duplex. CEAP C2 (small varicosities), Ep, As, Pn/Pr borderline.

2. No procedural indication at this time. Begin conservative management: leg elevation 3–4 times daily, daily walking, calf raises at the classroom desk, weight management, ibuprofen PRN. Compression stockings recommended again; patient prefers to try the other measures first through the holidays and will return in 2 months to be fitted if symptoms persist.

3. Return 2 months. Repeat duplex in 1–2 years or sooner if symptoms progress.

Orders:

• Return 2 months

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	50 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99213	Established patient, low complexity
I83.812	Varicose veins of left lower extremity with pain

Electronically signed: Dr. Elena Vasquez, MD, FACS — December 5, 2023 12:13

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: January 9, 2024 | Urgent Care Visit

Encounter	#4 of 40
Practice	Carrollwood Urgent Care — Urgent Care
Location	14030 N Dale Mabry Hwy | Tampa, FL 33618 | (813) 555-0193 | NPI: 1407732915
Attending	Dr. Anita Shah, MD
Encounter Type	Urgent Care Visit
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 55
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Facial pressure, nasal congestion and purulent drainage x 11 days.

History of Present Illness:

55-year-old female with 11 days of nasal congestion, thick yellow-green drainage, left maxillary pressure and low-grade fever to 100.4 °F that improved after day 5 then worsened again on day 9 ('double worsening'). Tried oral pseudoephedrine, saline rinses and acetaminophen with partial relief. No dental pain, no visual change, no severe headache. Non-smoker. No recent antibiotics.

Chronic issues stable; she mentions her vascular surgeon has her elevating the left leg for varicose veins; not a concern today.

Vital Signs:

Blood Pressure	126/86 mmHg, seated, left arm
Heart Rate	67 bpm, regular
Temperature	97.9 °F oral
Respiratory Rate	16 /min
SpO2	97% on room air
Weight	174 lb
Height	64 in
BMI	29.9 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.

Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Mildly uncomfortable, non-toxic.
HEENT	Left maxillary sinus tenderness to percussion. Nasal mucosa erythematous and edematous with purulent discharge from the left middle meatus. Tympanic membranes normal. Oropharynx with post-nasal drip.
Neck	No lymphadenopathy, no meningismus.
Lungs	Clear bilaterally.
Cardiovascular	Regular, no murmur.

Assessment & Plan:

1. Acute bacterial rhinosinusitis, left maxillary, meeting criteria for antibiotics (duration > 10 days with double worsening). Amoxicillin-clavulanate 875/125 mg BID x 7 days. Continue saline irrigation; intranasal fluticasone. Return if fever > 102 °F, periorbital swelling or no improvement in 72 hours.

2. Follow up with PCP as scheduled.

Orders:

• Amoxicillin-clavulanate 875/125 mg, #14, BID

• Fluticasone nasal spray OTC

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	50 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Amoxicillin-clavulanate	875/125 mg	BID x 7 days	Acute bacterial sinusitis

Changes since last encounter:

• STARTED Amoxicillin-clavulanate 875/125 mg BID x 7 days — Acute bacterial sinusitis (01/09/2024)

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Return to urgent care or go to the emergency department for worsening symptoms, high fever, difficulty breathing, chest pain, or new confusion.

• Follow up with your primary care physician within 1–2 weeks. A copy of this visit has been sent to Dr. Priya Raman.

• Rest, fluids, and over-the-counter symptom relief as discussed. Complete the full course of any prescribed antibiotic.

• Work note provided on request.

Encounter Coding:

Code	Description
99213	Urgent care visit, low complexity
J01.00	Acute maxillary sinusitis

Electronically signed: Dr. Anita Shah, MD — January 9, 2024 11:18

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: January 23, 2024 | Chronic Care Follow-Up (hypertension)

Encounter	#5 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Chronic Care Follow-Up (hypertension)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 55
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Hypertension follow-up; home readings elevated.

History of Present Illness:

Home blood pressure log over 3 weeks averages 142/88 (range 136–151 / 84–93), up from 120s–130s last year. She has gained 4 lb since October (holidays, less walking because her left leg aches by evening). No headache, chest pain or visual change. Takes losartan 50 mg daily without missed doses. Sodium intake up with school cafeteria lunches. Sinusitis last month resolved with antibiotics.

Left leg: continuing elevation and walking per vascular surgery; has not yet started compression stockings; has a follow-up with Dr. Vasquez next month.

Vital Signs:

Blood Pressure	129/79 mmHg, seated, left arm
Heart Rate	75 bpm, regular
Temperature	98.1 °F oral
Respiratory Rate	14 /min
SpO2	99% on room air
Weight	175 lb
Height	64 in
BMI	30.0 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.

Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Essential hypertension, above goal on home readings. Increase losartan to 100 mg daily. Repeat BMP in 2 weeks for potassium and creatinine. Reinforce DASH diet and sodium under 2 g. Home BP log, return 6–8 weeks.

2. Hypothyroidism, stable, TSH 10/2023 was 3.1. Continue levothyroxine 50 mcg.

3. Hyperlipidemia, newly flagged: LDL 158 on 10/2023 labs. Discussed lifestyle; recheck lipids in 4 months and decide on a statin.

4. Varicose veins left leg — followed by vascular surgery.

Orders:

• BMP in 2 weeks

• Losartan 100 mg daily, #90

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism

Medication	Dose	Frequency	Indication
Ibuprofen	400 mg	PRN	Left leg aching

Changes since last encounter:

• STOPPED Losartan 50 mg (01/23/2024)

• STARTED Losartan 100 mg Daily — Hypertension (01/23/2024)

• STOPPED Amoxicillin-clavulanate 875/125 mg (01/16/2024)

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
I10	Essential hypertension
E78.5	Hyperlipidemia

Electronically signed: Dr. Priya Raman, MD — January 23, 2024 16:56

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: February 13, 2024 | Follow-Up (conservative management)

Encounter	#6 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (conservative management)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 55
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Persistent left leg heaviness and evening swelling despite elevation and walking.

History of Present Illness:

Returns 2 months after starting elevation, walking and calf exercises. She reports partial compliance with elevation (most evenings) and walking (3 nights per week). Symptoms persist: heaviness and aching by early afternoon on school days, evening ankle swelling that now sometimes leaves a sock line, and new intermittent night cramps in the left calf about once a week. Ibuprofen 2–3 times per week. No ulcer, no bleeding, no thrombophlebitis.

She is now willing to try compression. She was measured by our fitter this morning (ankle circumference 23 cm, calf 38 cm) and fitted with knee-high graduated compression stockings, 20–30 mmHg, size medium-regular.

Patient fitted today, February 13, 2024, with 20–30 mmHg graduated knee-high compression stockings for the left leg; instructed to wear them daily from morning until bedtime. Start date of compression therapy: 02/13/2024.

She asks about the long term; we discussed that if a 3-month trial of compression plus the other measures does not control symptoms, and if reflux on repeat duplex meets threshold, endovenous ablation would be an option.

Vital Signs:

Blood Pressure	124/85 mmHg, seated, left arm
Heart Rate	70 bpm, regular
Temperature	98.4 °F oral
Respiratory Rate	15 /min
SpO2	99% on room air
Weight	172 lb
Height	64 in
BMI	29.5 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.

Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Medial calf varicosities in the GSV distribution, unchanged in extent from December. Mild pitting ankle edema at 1:15 pm. No pigmentation, no eczema, no ulcer. Calf soft, non-tender.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic left lower extremity varicose veins / early superficial venous insufficiency, CEAP C2 with evening edema (C2–C3), conservative management.

2. Compression therapy initiated today (20–30 mmHg graduated knee-high, daily) in addition to elevation, walking and ibuprofen PRN. Written stocking care instructions provided. Return in 3 months to assess response; sooner for new symptoms.

Orders:

• Graduated compression stockings, knee-high, 20–30 mmHg, left, 2 pairs (A6531)

• Return 3 months

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99213	Established patient, low complexity
I83.812	Varicose veins of left lower extremity with pain
I87.2	Venous insufficiency (chronic) (peripheral)

Electronically signed: Dr. Elena Vasquez, MD, FACS — February 13, 2024 10:23

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: March 19, 2024 | Physical Therapy Initial Evaluation

Encounter	#7 of 40
Practice	Tampa Bay Physical Therapy & Rehabilitation — Physical Therapy
Location	4415 W Gandy Blvd | Tampa, FL 33611 | (813) 555-0188 | NPI: 1821456730
Attending	Kevin Nunes, PT, DPT
Encounter Type	Physical Therapy Initial Evaluation
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Right anterior knee pain x 6 weeks, worse on stairs. Referred by Dr. Raman.

History of Present Illness:

56-year-old female teacher with 6 weeks of right anterior knee pain, insidious onset after starting a stair-climbing program at school in January. Pain 4/10 on stairs and after prolonged sitting ('theater sign'), 1/10 at rest. No locking, giving way or effusion. Right knee X-ray 03/12/2024 reviewed: no fracture, mild patellofemoral joint space narrowing.

Relevant history: left leg varicose veins managed by vascular surgery with compression stockings; patient wears a left knee-high stocking and asks that exercises not require removing it.

Vital Signs:

Blood Pressure	141/84 mmHg, seated, left arm
Heart Rate	73 bpm, regular
Temperature	98.0 °F oral
Respiratory Rate	14 /min
SpO2	98% on room air
Weight	170 lb
Height	64 in
BMI	29.2 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Observation	Mild genu valgum on single-leg squat right; no effusion.
Range of Motion	Right knee 0–135°, equal to left.
Strength	Right hip abduction 4-/5, quadriceps 4/5, left 5/5 throughout.
Special Tests	Patellar grind positive, McMurray negative, Lachman negative.
Palpation	Tenderness medial patellar facet. No joint line tenderness.
Left Lower Extremity	Wearing a 20–30 mmHg knee-high compression stocking per patient report; not removed for examination.

Treatment Log (today's session):

Exercise	Sets x Reps	Load / Band	Tolerance
Clamshell	3 x 12	Green band	No pain
Straight leg raise	3 x 10	Body weight	No pain
Wall sit 30 s	3	Body weight	2/10
Step-down 4 in	2 x 8	Body weight	3/10

Assessment & Plan:

1. Patellofemoral pain syndrome, right, with hip abductor weakness and quadriceps inhibition. Plan: 2x/week for 6 weeks — hip and quadriceps strengthening, patellar mobilization, taping, home program. Goals: stairs pain-free (0–1/10) in 6 weeks, single-leg squat with neutral knee alignment.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Home exercise program as demonstrated; 1 set of 10–15 repetitions, twice daily, within a pain-free range.

• Ice 15 minutes after exercise if sore. Heat before stretching is acceptable.

• Pain that persists more than 2 hours after exercise means the dose was too high; reduce repetitions and inform the therapist.

• Wear supportive footwear; avoid barefoot walking on hard surfaces.

Encounter Coding:

Code	Description
97161	PT evaluation, low complexity
97110	Therapeutic exercise
M22.2X1	Patellofemoral disorder, right knee

Electronically signed: Kevin Nunes, PT, DPT — March 19, 2024 9:28

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: April 2, 2024 | Physical Therapy Treatment (visit 3 of 12)

Encounter	#8 of 40
Practice	Tampa Bay Physical Therapy & Rehabilitation — Physical Therapy
Location	4415 W Gandy Blvd | Tampa, FL 33611 | (813) 555-0188 | NPI: 1821456730
Attending	Kevin Nunes, PT, DPT
Encounter Type	Physical Therapy Treatment (visit 3 of 12)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Right knee pain, improving.

History of Present Illness:

Reports stair pain down to 2/10. Doing the home program 5 days per week. No new complaints.

Notes she keeps her left compression stocking on for sessions; no problems with it.

Vital Signs:

Blood Pressure	125/77 mmHg, seated, left arm
Heart Rate	81 bpm, regular
Temperature	98.2 °F oral
Respiratory Rate	16 /min
SpO2	97% on room air
Weight	171 lb
Height	64 in
BMI	29.3 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.

Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Strength	Right hip abduction 4/5, quadriceps 4+/5.
Observation	Improved knee alignment on step-down.

Treatment Log (today's session):

Exercise	Sets x Reps	Load / Band	Tolerance
Clamshell	3 x 15	Blue band	No pain
Lateral step-up 6 in	3 x 10	Body weight	1/10
Single-leg balance	3 x 30 s	Foam pad	No pain
Bike warm-up	8 min	Level 3	No pain

Assessment & Plan:

1. Progressing well. Advanced to lateral step-ups and single-leg balance. Continue 2x/week.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Home exercise program as demonstrated; 1 set of 10–15 repetitions, twice daily, within a pain-free range.

• Ice 15 minutes after exercise if sore. Heat before stretching is acceptable.

• Pain that persists more than 2 hours after exercise means the dose was too high; reduce repetitions and inform the therapist.

• Wear supportive footwear; avoid barefoot walking on hard surfaces.

Encounter Coding:

Code	Description
97110	Therapeutic exercise
97140	Manual therapy
M22.2X1	Patellofemoral disorder, right knee

Electronically signed: Kevin Nunes, PT, DPT — April 2, 2024 14:06

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: April 16, 2024 | Physical Therapy Treatment (visit 6 of 12)

Encounter	#9 of 40
Practice	Tampa Bay Physical Therapy & Rehabilitation — Physical Therapy
Location	4415 W Gandy Blvd | Tampa, FL 33611 | (813) 555-0188 | NPI: 1821456730
Attending	Kevin Nunes, PT, DPT
Encounter Type	Physical Therapy Treatment (visit 6 of 12)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Right knee pain, nearly resolved.

History of Present Illness:

Stair pain 0–1/10. Returned to full stair program at school. Home program adherence good.

Patient comments that the left-leg compression stocking she wears daily for her vein condition is 'getting hot' as spring arrives but she is still putting it on each morning.

Vital Signs:

Blood Pressure	128/81 mmHg, seated, left arm
Heart Rate	72 bpm, regular
Temperature	98.4 °F oral
Respiratory Rate	14 /min
SpO2	99% on room air
Weight	172 lb
Height	64 in
BMI	29.5 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.

Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Strength	Right hip abduction 5/5, quadriceps 5/5.
Functional	Single-leg squat with neutral alignment, 10 reps pain-free.

Treatment Log (today's session):

Exercise	Sets x Reps	Load / Band	Tolerance
Lateral step-up 8 in	3 x 12	Body weight	0/10
Single-leg RDL	3 x 8	10 lb	0/10
Split squat	3 x 10	Body weight	1/10

Assessment & Plan:

1. Goals met. Discharge to independent home program with written progression. Re-evaluate PRN.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Home exercise program as demonstrated; 1 set of 10–15 repetitions, twice daily, within a pain-free range.

• Ice 15 minutes after exercise if sore. Heat before stretching is acceptable.

• Pain that persists more than 2 hours after exercise means the dose was too high; reduce repetitions and inform the therapist.

• Wear supportive footwear; avoid barefoot walking on hard surfaces.

Encounter Coding:

Code	Description
97110	Therapeutic exercise
M22.2X1	Patellofemoral disorder, right knee

Electronically signed: Kevin Nunes, PT, DPT — April 16, 2024 10:44

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: May 21, 2024 | Chronic Care Follow-Up (hypertension, lipids)

Encounter	#10 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Chronic Care Follow-Up (hypertension, lipids)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Blood pressure recheck and lipid results.

History of Present Illness:

Home BP log averages 128/80 on losartan 100 mg. BMP 02/2024: potassium 4.3, creatinine 0.82. No side effects. Fasting lipid panel 05/14/2024: total cholesterol 238, LDL 161, HDL 52, triglycerides 128. 10-year ASCVD risk 8.9%. She finished physical therapy for the right knee with good result. Weight stable.

Left leg: continues to follow with vascular surgery; wearing the compression stocking most days, says it is uncomfortable now that the weather is warm.

Vital Signs:

Blood Pressure	126/80 mmHg, seated, left arm
Heart Rate	75 bpm, regular
Temperature	98.0 °F oral
Respiratory Rate	17 /min
SpO2	98% on room air
Weight	170 lb
Height	64 in
BMI	29.2 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Essential hypertension, at goal on losartan 100 mg. Continue.

2. Hyperlipidemia, LDL 161 despite 4 months of lifestyle change, intermediate risk. Shared decision: start atorvastatin 20 mg nightly. Lipids and hepatic panel in 8–12 weeks.

3. Hypothyroidism, stable. Varicose veins, left — per vascular surgery.

4. Return 3 months.

Orders:

• Atorvastatin 20 mg nightly, #90

• Lipid panel and hepatic function panel in 8–12 weeks

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia

Changes since last encounter:

• STARTED Atorvastatin 20 mg Nightly — Hyperlipidemia (05/21/2024)

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
I10	Essential hypertension
E78.5	Hyperlipidemia

Electronically signed: Dr. Priya Raman, MD — May 21, 2024 9:49

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: July 16, 2024 | New Patient — Full Skin Examination

Encounter	#11 of 40
Practice	Bayshore Dermatology Group — Dermatology
Location	3100 W Kennedy Blvd, Suite 205 | Tampa, FL 33609 | (813) 555-0177 | NPI: 1639120084
Attending	Dr. Marcus Bell, MD
Encounter Type	New Patient — Full Skin Examination
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Annual skin check requested; several 'spots' on the face and back; itchy hands.

History of Present Illness:

56-year-old female, Fitzpatrick type III, with extensive childhood sun exposure in Florida, no prior skin cancer, no family history of melanoma. Requests a full skin examination. Reports a rough scaly spot on the left temple present for several months and several raised brown 'barnacles' on the back. Also 3 months of dry, itchy, cracking skin on the dorsal hands and finger webs, worse with frequent hand-washing and classroom sanitizer.

Review of lower extremities: she has varicose veins on the left leg followed by vascular surgery and was prescribed compression stockings earlier this year.

She mentions she "stopped wearing stockings for the summer" — she took them off at the beginning of May because of the heat and has not worn them since; she plans to resume when school starts.

Medications: losartan, levothyroxine, atorvastatin, ibuprofen PRN. No photosensitizing agents.

Vital Signs:

Blood Pressure	138/85 mmHg, seated, left arm
Heart Rate	73 bpm, regular
Temperature	97.9 °F oral
Respiratory Rate	17 /min
SpO2	97% on room air
Weight	174 lb
Height	64 in
BMI	29.9 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.

Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Skin — Face	6 mm rough erythematous scaly papule left temple consistent with actinic keratosis. Solar lentigines bilateral cheeks.
Skin — Trunk	Multiple 4–10 mm stuck-on waxy brown papules and plaques upper back and chest, seborrheic keratoses. No atypical nevi.
Skin — Hands	Dorsal hands and finger webs with xerosis, fine scale, fissuring of the 3rd and 4th webs; no vesicles.
Skin — Lower Extremities	Left medial calf varicosities, small, without hemosiderin pigmentation, eczema, atrophie blanche or ulceration. Mild ankle edema. Right leg unremarkable. Nails normal.
Lymph	No cervical, axillary or inguinal adenopathy.

Assessment & Plan:

1. Actinic keratosis, left temple, solitary, 6 mm. Cryotherapy with liquid nitrogen, 2 freeze–thaw cycles, performed today.

2. Seborrheic keratoses, back and chest, multiple, benign. Reassurance; no treatment.

3. Irritant hand dermatitis. Emollient regimen, barrier technique, avoid sanitizer when soap and water are available. If no better in 8 weeks, will add a mid-potency topical steroid.

4. Left lower extremity: varicose veins without stasis dermatitis, pigmentation or ulcer on today's skin exam; defer management to vascular surgery; encouraged her to resume compression as planned.

5. Return 4 months for hand dermatitis follow-up and AK site check; annual full skin exam thereafter.

Orders:

• Cryotherapy, 1 lesion (17000)

• Ceramide emollient OTC

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Daily broad-spectrum sunscreen SPF 30 or higher on exposed skin; reapply every 2 hours outdoors.

• Monthly self skin examination using the ABCDE rule; photograph any changing lesion and call the office.

• Bland emollient (petrolatum or ceramide cream) to hands twice daily; avoid fragranced soaps.

• Return sooner for any lesion that bleeds, grows, or does not heal within 4 weeks.

Encounter Coding:

Code	Description
99203	New patient, low complexity
17000	Destruction premalignant lesion, first
L57.0	Actinic keratosis
L82.1	Seborrheic keratosis
L24.9	Irritant contact dermatitis

Electronically signed: Dr. Marcus Bell, MD — July 16, 2024 11:21

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: August 6, 2024 | Urgent Care Visit

Encounter	#12 of 40
Practice	Carrollwood Urgent Care — Urgent Care
Location	14030 N Dale Mabry Hwy | Tampa, FL 33618 | (813) 555-0193 | NPI: 1407732915
Attending	Luis Herrera, PA-C
Encounter Type	Urgent Care Visit
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Red, warm, painful area on right forearm after insect bite 3 days ago.

History of Present Illness:

56-year-old female with a mosquito bite on the right volar forearm 3 days ago, now with 5 x 4 cm area of spreading redness, warmth, and tenderness, and streaking 2 cm proximally. No fever, no purulence, no fluctuance. No diabetes. Tetanus up to date (2019). Scratched the bite repeatedly.

Otherwise well. She notes she is back in the classroom pre-planning this week.

Vital Signs:

Blood Pressure	133/80 mmHg, seated, left arm
Heart Rate	68 bpm, regular
Temperature	98.2 °F oral
Respiratory Rate	14 /min
SpO2	97% on room air
Weight	171 lb
Height	64 in
BMI	29.3 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Afebrile, non-toxic.
Skin	Right volar forearm: 5 x 4 cm erythema, warmth, induration around a central excoriated papule; 2 cm lymphangitic streak; no fluctuance or abscess.
Lymph	No epitrochlear or axillary adenopathy.
Cardiovascular	Regular rate, no murmur.

Assessment & Plan:

1. Non-purulent cellulitis, right forearm, with early lymphangitis. Cephalexin 500 mg QID x 7 days. Margins marked; elevate; warm compresses. Return in 48 hours if the redness extends beyond the marked line or if fever develops.

Orders:

• Cephalexin 500 mg #28, QID x 7 days

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cephalexin	500 mg	QID x 7 days	Cellulitis, right forearm

Changes since last encounter:

• STARTED Cephalexin 500 mg QID x 7 days — Cellulitis, right forearm (08/06/2024)

Allergies: No known drug allergies.

Patient Instructions / After Visit Summary:

• Return to urgent care or go to the emergency department for worsening symptoms, high fever, difficulty breathing, chest pain, or new confusion.

• Follow up with your primary care physician within 1–2 weeks. A copy of this visit has been sent to Dr. Priya Raman.

• Rest, fluids, and over-the-counter symptom relief as discussed. Complete the full course of any prescribed antibiotic.

• Work note provided on request.

Encounter Coding:

Code	Description
99213	Urgent care visit, low complexity
L03.113	Cellulitis of right upper limb
W57.XXXA	Insect bite

Electronically signed: Luis Herrera, PA-C — August 6, 2024 14:48

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: September 17, 2024 | Follow-Up (conservative management)

Encounter	#13 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (conservative management)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Left leg heaviness, aching and swelling, worse with return to school.

History of Present Illness:

Patient returns later than planned (last seen 02/13/2024). She started 20–30 mmHg compression stockings on 02/13/2024 and wore them daily through April. She stopped wearing them at the beginning of May 2024 because of the heat and did not wear them through the summer. During the summer break, with less standing, symptoms were tolerable. Since the school year began in mid-August the heaviness, aching and evening ankle swelling are back and are worse than last spring; she now has night cramps 2–3 times per week and the leg 'throbs' by the last period. Ibuprofen 3–4 times per week.

Functional impact: she sits on a stool to teach the last two periods, has stopped evening walks entirely on school days, and asked the principal to be excused from car-line duty, which requires 40 minutes of standing.

No ulcer, no bleeding, no thrombophlebitis. No DVT. Cellulitis of the right forearm last month resolved; she developed a mild rash on cephalexin, now listed as an allergy.

Compression therapy restarted today, 09/17/2024: 20–30 mmHg graduated knee-high stockings, left leg, daily from morning until bedtime, with a re-fitting (new pair dispensed, same size). She agrees to wear them without interruption, including the summer, and to keep a log.

Discussed that for insurance purposes a continuous trial of at least 3 months is required and that the interrupted spring trial (02/13/2024 to early May 2024) does not count as a completed trial.

Vital Signs:

Blood Pressure	142/81 mmHg, seated, left arm
Heart Rate	75 bpm, regular
Temperature	97.9 °F oral
Respiratory Rate	16 /min
SpO2	97% on room air
Weight	174 lb
Height	64 in
BMI	29.9 kg/m²
Pain Score	4/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.

Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Medial calf and distal thigh varicosities in the GSV distribution, more prominent than in February; mild pitting edema at the ankle at 3:30 pm. No pigmentation, eczema or ulcer. Calf soft, non-tender.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic left lower extremity varicose veins and superficial venous insufficiency, CEAP C2–C3 (evening edema), with worsening symptoms after an interrupted compression trial.

2. Compression therapy resumed today (restart date 09/17/2024), 20–30 mmHg graduated knee-high, daily, plus elevation, walking, ibuprofen PRN. Return 3 months with the compliance log. If symptoms persist at 3 months of uninterrupted compression, repeat the venous duplex and discuss endovenous ablation.

Orders:

• Graduated compression stockings, knee-high, 20–30 mmHg, left, 2 pairs (A6531)

• Return 3 months

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	50 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia

Changes since last encounter:

• STOPPED Cephalexin 500 mg (08/13/2024)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
I83.812	Varicose veins of left lower extremity with pain
I87.2	Venous insufficiency (chronic) (peripheral)

Electronically signed: Dr. Elena Vasquez, MD, FACS — September 17, 2024 11:42

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: October 15, 2024 | Annual Physical / Preventive Visit

Encounter	#14 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Annual Physical / Preventive Visit
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Annual physical.

History of Present Illness:

Annual visit. Hypertension at goal on losartan 100 mg (home average 126/78). Lipids 08/2024 on atorvastatin 20 mg: LDL 96, ALT normal. TSH 09/2024: 5.9 (elevated) with free T4 low-normal, mild fatigue and cold intolerance. Vitamin D 09/2024: 22 ng/mL. Mammogram 11/2023 BI-RADS 1. Influenza vaccine today.

Left leg: she restarted compression stockings on 09/17/2024 under vascular surgery and reports wearing them every day since; symptoms still present by late afternoon. Followed by Dr. Vasquez.

Vital Signs:

Blood Pressure	129/78 mmHg, seated, left arm
Heart Rate	74 bpm, regular
Temperature	98.3 °F oral
Respiratory Rate	16 /min
SpO2	98% on room air
Weight	176 lb
Height	64 in
BMI	30.2 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Hypothyroidism, under-replaced (TSH 5.9). Increase levothyroxine to 75 mcg daily; TSH in 8 weeks.

2. Vitamin D insufficiency: cholecalciferol 2000 IU daily.

3. Hypertension, at goal. Hyperlipidemia, at goal on atorvastatin 20 mg.

4. Varicose veins left leg with chronic venous insufficiency, symptomatic, on compression per vascular surgery.

5. Preventive: influenza vaccine given. Mammogram due 11/2024 — ordered. Return 6 months.

Orders:

• Levothyroxine 75 mcg daily, #90

• Cholecalciferol 2000 IU daily OTC

• TSH in 8 weeks

• Screening mammogram

• Influenza vaccine, administered

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension

Medication	Dose	Frequency	Indication
Levothyroxine	75 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Changes since last encounter:

• STOPPED Levothyroxine 50 mcg (10/15/2024)

• STARTED Levothyroxine 75 mcg Daily — Hypothyroidism (10/15/2024)

• STARTED Cholecalciferol (Vitamin D3) 2000 IU Daily — Vitamin D insufficiency (10/15/2024)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99396	Preventive visit, established
E03.9	Hypothyroidism
E55.9	Vitamin D deficiency
I10	Essential hypertension

Electronically signed: Dr. Priya Raman, MD — October 15, 2024 12:58

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: November 12, 2024 | Follow-Up

Encounter	#15 of 40
Practice	Bayshore Dermatology Group — Dermatology
Location	3100 W Kennedy Blvd, Suite 205 | Tampa, FL 33609 | (813) 555-0177 | NPI: 1639120084
Attending	Dr. Marcus Bell, MD
Encounter Type	Follow-Up
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Hand dermatitis not improved; AK site check.

History of Present Illness:

Hand dermatitis persists despite 4 months of emollients and barrier measures; worse with the school year. Itching interferes with grading papers. AK site on left temple healed without recurrence.

She is wearing her left compression stocking today ('every day since September').

Vital Signs:

Blood Pressure	135/86 mmHg, seated, left arm
Heart Rate	73 bpm, regular
Temperature	97.8 °F oral
Respiratory Rate	16 /min
SpO2	99% on room air
Weight	169 lb
Height	64 in
BMI	29.0 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Skin — Hands	Dorsal hands with lichenified plaques, fissures 3rd/4th webs, no vesicles, no nail changes.
Skin — Face	Left temple: faint hypopigmented macule at prior cryotherapy site; no residual AK.
Skin — Legs	Left knee-high compression stocking in place; not removed.

Assessment & Plan:

1. Irritant hand dermatitis, persistent. Add triamcinolone 0.1% cream BID PRN to the hands for up to 2 weeks at a time; continue emollients.

2. Actinic keratosis, left temple, resolved after cryotherapy. Return 9 months for annual full skin exam.

Orders:

• Triamcinolone 0.1% cream 30 g, BID PRN

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Triamcinolone 0.1% cream	Topical	BID PRN	Hand dermatitis

Changes since last encounter:

• STARTED Triamcinolone 0.1% cream Topical BID PRN — Hand dermatitis (11/12/2024)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Daily broad-spectrum sunscreen SPF 30 or higher on exposed skin; reapply every 2 hours outdoors.

• Monthly self skin examination using the ABCDE rule; photograph any changing lesion and call the office.

• Bland emollient (petrolatum or ceramide cream) to hands twice daily; avoid fragranced soaps.

• Return sooner for any lesion that bleeds, grows, or does not heal within 4 weeks.

Encounter Coding:

Code	Description
99213	Established patient, low complexity
L24.9	Irritant contact dermatitis

Electronically signed: Dr. Marcus Bell, MD — November 12, 2024 13:14

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: December 17, 2024 | Follow-Up (3-month compression check)

Encounter	#16 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (3-month compression check)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Left leg heaviness, aching, swelling and night cramps, unchanged on daily compression.

History of Present Illness:

Three months since compression was restarted on 09/17/2024. Compliance log reviewed: stocking worn daily, 6–7 days per week, no missed weeks; she removes it at bedtime. Elevation most evenings, walking 2 nights per week. Ibuprofen 3 times per week.

Symptoms are unchanged: aching and heaviness by early afternoon on every teaching day, visible ankle swelling by evening, night cramps 2 times per week. Compression 'helps a little in the morning' but by afternoon the leg aches regardless. No ulcer, no bleeding, no phlebitis.

Functional impact unchanged from September: teaching from a stool, no evening walks on school days, excused from car-line duty.

Vital Signs:

Blood Pressure	133/85 mmHg, seated, left arm
Heart Rate	76 bpm, regular
Temperature	98.3 °F oral
Respiratory Rate	15 /min
SpO2	98% on room air
Weight	176 lb
Height	64 in
BMI	30.2 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.

Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Medial calf and distal thigh varicosities in the GSV distribution, tortuous, unchanged from September. 1+ pitting ankle edema at 2:00 pm despite the stocking being worn until the exam. No pigmentation, eczema or ulcer.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic left lower extremity superficial venous insufficiency, CEAP C3 (edema), with persistent symptoms after 3 months of uninterrupted 20–30 mmHg compression (09/17/2024 to 12/17/2024) plus elevation, exercise and NSAIDs; no improvement.

2. Patient prefers to continue conservative management through the school year and revisit after summer, as she is the sole caregiver for her mother and cannot schedule a procedure now. Continue compression without interruption. Return 3 months. Repeat duplex before any procedure.

Orders:

• Continue compression 20–30 mmHg daily

• Return 3 months

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Triamcinolone 0.1% cream	Topical	BID PRN	Hand dermatitis

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99213	Established patient, low complexity
I83.812	Varicose veins of left lower extremity with pain
I87.2	Venous insufficiency (chronic) (peripheral)

Electronically signed: Dr. Elena Vasquez, MD, FACS — December 17, 2024 12:19

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: January 14, 2025 | Urgent Care Visit

Encounter	#17 of 40
Practice	Carrollwood Urgent Care — Urgent Care
Location	14030 N Dale Mabry Hwy | Tampa, FL 33618 | (813) 555-0193 | NPI: 1407732915
Attending	Dr. Omar Siddiqui, DO
Encounter Type	Urgent Care Visit
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Cough x 9 days, now productive, with chest tightness.

History of Present Illness:

56-year-old non-smoker with 9 days of cough after an upper respiratory infection, now productive of yellow sputum, with chest tightness on coughing, no hemoptysis, low-grade fever to 100.1 °F on day 3 only, no dyspnea at rest. Many sick children in the classroom. No wheeze history, no asthma.

Wearing a compression stocking on the left leg for varicose veins; denies calf pain or unilateral swelling beyond her usual.

Vital Signs:

Blood Pressure	139/82 mmHg, seated, left arm
Heart Rate	75 bpm, regular
Temperature	97.8 °F oral
Respiratory Rate	15 /min
SpO2	99% on room air
Weight	169 lb
Height	64 in
BMI	29.0 kg/m²
Pain Score	2/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Afebrile, non-toxic, speaking full sentences.
Lungs	Scattered rhonchi that clear with cough; no focal crackles, no wheeze. SpO2 98%.
Cardiovascular	Regular, no murmur.
HEENT	Mild pharyngeal erythema.

Assessment & Plan:

1. Acute bronchitis, likely viral. Chest X-ray today without infiltrate (see report). Benzonatate 100 mg TID PRN x 7 days, honey, fluids, rest. No antibiotics indicated. Return for fever, dyspnea or symptoms beyond 3 weeks.

Orders:

• Chest X-ray, PA and lateral (71046)

• Benzonatate 100 mg #21

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Triamcinolone 0.1% cream	Topical	BID PRN	Hand dermatitis
Benzonatate	100 mg	TID PRN x 7 days	Cough, acute bronchitis

Changes since last encounter:

• STARTED Benzonatate 100 mg TID PRN x 7 days — Cough, acute bronchitis (01/14/2025)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Return to urgent care or go to the emergency department for worsening symptoms, high fever, difficulty breathing, chest pain, or new confusion.

• Follow up with your primary care physician within 1–2 weeks. A copy of this visit has been sent to Dr. Priya Raman.

• Rest, fluids, and over-the-counter symptom relief as discussed. Complete the full course of any prescribed antibiotic.

• Work note provided on request.

Encounter Coding:

Code	Description
99213	Urgent care visit, low complexity
71046	Chest X-ray 2 views
J20.9	Acute bronchitis

Electronically signed: Dr. Omar Siddiqui, DO — January 14, 2025 13:35

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: February 11, 2025 | Chronic Care Follow-Up

Encounter	#18 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Chronic Care Follow-Up
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 56
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Thyroid recheck; blood pressure; medication refills.

History of Present Illness:

TSH 12/2024 on levothyroxine 75 mcg: 2.4, fatigue improved. Home BP average 124/78. Hand dermatitis resolved with triamcinolone; stopped using it. Bronchitis last month resolved. Mammogram 11/2024 BI-RADS 2, benign.

Left leg: wearing compression stocking daily since September per vascular surgery; still symptomatic by afternoon; she is deferring a procedure because she is caring for her mother.

Vital Signs:

Blood Pressure	126/79 mmHg, seated, left arm
Heart Rate	74 bpm, regular
Temperature	98.2 °F oral
Respiratory Rate	15 /min
SpO2	97% on room air
Weight	171 lb
Height	64 in
BMI	29.3 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Hypothyroidism, euthyroid on 75 mcg. Continue; TSH annually.

2. Hypertension, at goal. Hyperlipidemia, at goal.

3. Hand dermatitis resolved; triamcinolone discontinued from the medication list.

4. Varicose veins/chronic venous insufficiency, left, symptomatic on compression — per vascular surgery. Refills sent. Return 6 months.

Orders:

- Losartan 100 mg #90
- Levothyroxine 75 mcg #90
- Atorvastatin 20 mg #90

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia

Medication	Dose	Frequency	Indication
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Changes since last encounter:

• STOPPED Triamcinolone 0.1% cream Topical (02/11/2025)

• STOPPED Benzonatate 100 mg (01/21/2025)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99213	Established patient, low complexity
E03.9	Hypothyroidism
I10	Essential hypertension

Electronically signed: Dr. Priya Raman, MD — February 11, 2025 14:51

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: March 18, 2025 | Follow-Up (conservative management)

Encounter	#19 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (conservative management)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Persistent left leg symptoms on compression; discussion of ablation.

History of Present Illness:

Six months of uninterrupted 20–30 mmHg compression (since 09/17/2024), log reviewed, daily use confirmed. Symptoms persist: aching and heaviness daily by early afternoon, evening ankle edema, night cramps 1–2 per week, new itching over the medial calf. Ibuprofen 3 times per week. No ulcer, bleeding or phlebitis.

ADLs: continues to teach from a stool, excused from standing duties, has not resumed evening walks, and reports she can no longer do her Saturday grocery and errand round without a mid-trip sit-down. She remains her mother's caregiver and asks to defer any procedure until after the summer when respite care is arranged.

Vital Signs:

Blood Pressure	124/78 mmHg, seated, left arm
Heart Rate	77 bpm, regular
Temperature	97.8 °F oral
Respiratory Rate	14 /min
SpO2	99% on room air
Weight	169 lb
Height	64 in
BMI	29.0 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.

Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Tortuous varicosities medial calf and distal medial thigh in the GSV distribution. 1+ pitting ankle edema at 3:00 pm with stocking worn until arrival. Mild dry scale medial calf without pigmentation or eczema. No ulcer.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic left lower extremity superficial venous insufficiency, CEAP C3, persistent after 6 months of continuous compression and conservative measures without improvement.

2. Endovenous ablation discussed in detail (radiofrequency vs laser, risks, benefits, alternatives); patient wishes to defer until fall 2025 for family reasons. Continue compression without interruption. Return 4 months. Repeat left venous reflux duplex to be ordered within 3 months of any planned procedure.

Orders:

- Continue compression 20–30 mmHg daily
- Return 4 months

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension

Medication	Dose	Frequency	Indication
Levothyroxine	75 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
I83.812	Varicose veins of left lower extremity with pain
I87.2	Venous insufficiency (chronic) (peripheral)

Electronically signed: Dr. Elena Vasquez, MD, FACS — March 18, 2025 13:56

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: April 22, 2025 | Physical Therapy Initial Evaluation

Encounter	#20 of 40
Practice	Tampa Bay Physical Therapy & Rehabilitation — Physical Therapy
Location	4415 W Gandy Blvd | Tampa, FL 33611 | (813) 555-0188 | NPI: 1821456730
Attending	Kevin Nunes, PT, DPT
Encounter Type	Physical Therapy Initial Evaluation
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Right shoulder pain with overhead reaching x 2 months. Referred by Dr. Raman.

History of Present Illness:

57-year-old right-handed teacher with 2 months of right lateral shoulder pain reaching overhead to the whiteboard and lifting boxes during a classroom move. Night pain lying on the right side. No trauma, no numbness. Pain 5/10 with overhead reach, 1/10 at rest.

Left compression stocking worn daily per vascular surgery; not relevant to today's treatment.

Vital Signs:

Blood Pressure	141/77 mmHg, seated, left arm
Heart Rate	80 bpm, regular
Temperature	98.3 °F oral
Respiratory Rate	17 /min
SpO2	98% on room air
Weight	176 lb
Height	64 in
BMI	30.2 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Range of Motion	Right shoulder flexion 160° (painful arc 90–120°), abduction 150°, ER 80°, IR to L1. Left full.
Strength	Right supraspinatus 4/5 painful, ER 4/5, scapular retractors 4-/5.
Special Tests	Hawkins-Kennedy positive, Neer positive, empty can painful, drop arm negative.

Treatment Log (today's session):

Exercise	Sets x Reps	Load / Band	Tolerance
Scapular retraction	3 x 15	Red band	No pain
Sidelying ER	3 x 12	2 lb	2/10
Sleeper stretch	3 x 30 s	—	Stretch only
Wall slides	3 x 10	Body weight	1/10

Assessment & Plan:

1. Right subacromial pain syndrome / rotator cuff tendinopathy with scapular dyskinesis. Plan 2x/week x 6 weeks: scapular stabilization, rotator cuff strengthening, posterior capsule stretching, postural training.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Home exercise program as demonstrated; 1 set of 10–15 repetitions, twice daily, within a pain-free range.

• Ice 15 minutes after exercise if sore. Heat before stretching is acceptable.

• Pain that persists more than 2 hours after exercise means the dose was too high; reduce repetitions and inform the therapist.

• Wear supportive footwear; avoid barefoot walking on hard surfaces.

Encounter Coding:

Code	Description
97161	PT evaluation, low complexity
M75.41	Impingement syndrome of right shoulder

Electronically signed: Kevin Nunes, PT, DPT — April 22, 2025 12:01

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: May 6, 2025 | Physical Therapy Treatment (visit 4 of 12)

Encounter	#21 of 40
Practice	Tampa Bay Physical Therapy & Rehabilitation — Physical Therapy
Location	4415 W Gandy Blvd | Tampa, FL 33611 | (813) 555-0188 | NPI: 1821456730
Attending	Kevin Nunes, PT, DPT
Encounter Type	Physical Therapy Treatment (visit 4 of 12)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Right shoulder pain, improving.

History of Present Illness:

Overhead pain down to 2/10; night pain resolved. Home program 5–6 days per week.

Mentions her left leg aches by the end of the school day despite her stocking; unrelated to today's treatment.

Vital Signs:

Blood Pressure	125/81 mmHg, seated, left arm
Heart Rate	71 bpm, regular
Temperature	97.6 °F oral
Respiratory Rate	15 /min
SpO2	97% on room air
Weight	168 lb
Height	64 in
BMI	28.8 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.

Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	Denies any history of deep vein thrombosis, pulmonary embolism or superficial thrombophlebitis. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Range of Motion	Right flexion 170°, painful arc diminished.
Strength	Supraspinatus 4+/5, ER 4+/5.

Treatment Log (today's session):

Exercise	Sets x Reps	Load / Band	Tolerance
Sidelying ER	3 x 15	3 lb	1/10
Prone Y/T/W	3 x 10	1 lb	No pain
Overhead press	3 x 10	5 lb	2/10
Rows	3 x 12	Blue band	No pain

Assessment & Plan:

1. Progressing. Advanced rotator cuff loading; added overhead press pattern. Continue 2x/week.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Ibuprofen	400 mg	PRN	Left leg aching
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Home exercise program as demonstrated; 1 set of 10–15 repetitions, twice daily, within a pain-free range.

• Ice 15 minutes after exercise if sore. Heat before stretching is acceptable.

• Pain that persists more than 2 hours after exercise means the dose was too high; reduce repetitions and inform the therapist.

• Wear supportive footwear; avoid barefoot walking on hard surfaces.

Encounter Coding:

Code	Description
97110	Therapeutic exercise
97140	Manual therapy
M75.41	Impingement syndrome of right shoulder

Electronically signed: Kevin Nunes, PT, DPT — May 6, 2025 8:39

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: June 10, 2025 | Urgent Care Visit

Encounter	#22 of 40
Practice	Carrollwood Urgent Care — Urgent Care
Location	14030 N Dale Mabry Hwy | Tampa, FL 33618 | (813) 555-0193 | NPI: 1407732915
Attending	Dr. Anita Shah, MD
Encounter Type	Urgent Care Visit
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Right calf pain and swelling x 2 days after a long car trip.

History of Present Illness:

57-year-old female with 2 days of right calf pain and swelling that began the day after a 9-hour car drive back from Atlanta (school-break trip). Pain is a deep ache, worse walking, right calf visibly larger than the left. No chest pain, no dyspnea, no hemoptysis. No trauma. Not on hormonal therapy. Non-smoker. Family history: mother had varicose veins; no family history of clots.

Of note, the LEFT leg is the one followed by vascular surgery for varicose veins (compression stocking worn daily); the current complaint is in the previously asymptomatic RIGHT leg. Wells score 3 (calf swelling > 3 cm, localized tenderness, recent immobilization): DVT likely.

Same-day venous duplex of the right lower extremity obtained at Carrollwood Imaging: acute deep vein thrombosis of the right posterior tibial vein (calf), non-occlusive, proximal veins patent. See report.

Treatment started today: apixaban 10 mg BID x 7 days, then 5 mg BID, planned duration 3 months (provoked distal DVT). First dose given in clinic. Ibuprofen stopped; acetaminophen for pain. PCP notified by phone and follow-up arranged in 1 week.

Vital Signs:

Blood Pressure	142/80 mmHg, seated, left arm
Heart Rate	74 bpm, regular
Temperature	98.1 °F oral
Respiratory Rate	14 /min
SpO2	99% on room air
Weight	175 lb
Height	64 in
BMI	30.0 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.

Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of acute right posterior tibial (calf) vein DVT diagnosed 06/10/2025, currently on apixaban. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Afebrile, comfortable at rest, SpO2 98% RA, HR 78.
Right Lower Extremity	Calf circumference 39.5 cm (left 36.0 cm, measured 10 cm below tibial tuberosity). Tenderness along the posterior tibial vein distribution; mild pitting edema at the ankle; no erythema; Homans equivocal. Pulses 2+.
Left Lower Extremity	Compression stocking in place; medial calf varicosities; mild chronic ankle edema; no tenderness.
Lungs	Clear.
Cardiovascular	Regular rate, no murmur.

Assessment & Plan:

1. Acute provoked deep vein thrombosis, right posterior tibial vein (calf DVT), after prolonged travel. Hemodynamically stable, no features of PE. Treated: apixaban started today 06/10/2025 (10 mg BID x 7 days, then 5 mg BID for 3 months total). Avoid NSAIDs. Ambulate. Return precautions for chest pain, dyspnea, bleeding, or extension of swelling. Follow up with PCP 1 week; vascular surgery informed.

Orders:

• Venous duplex, right lower extremity, STAT (93970)

• Apixaban 10 mg #14 then 5 mg #180

• CBC, BMP, PT/INR today

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism

Medication	Dose	Frequency	Indication
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Apixaban	10 mg	BID x 7 days (loading)	Acute DVT, right calf

Changes since last encounter:

• STOPPED Ibuprofen 400 mg (06/10/2025)

• STARTED Acetaminophen 500 mg PRN — Left leg aching (NSAIDs held with anticoagulation; continued by preference) (06/10/2025)

• STARTED Apixaban 10 mg BID x 7 days (loading) — Acute DVT, right calf (06/10/2025)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Return to urgent care or go to the emergency department for worsening symptoms, high fever, difficulty breathing, chest pain, or new confusion.

• Follow up with your primary care physician within 1–2 weeks. A copy of this visit has been sent to Dr. Priya Raman.

• Rest, fluids, and over-the-counter symptom relief as discussed. Complete the full course of any prescribed antibiotic.

• Work note provided on request.

Encounter Coding:

Code	Description
99214	Urgent care visit, moderate complexity
I82.451	Acute embolism and thrombosis of right posterior tibial vein
Z79.01	Long-term anticoagulant use

Electronically signed: Dr. Anita Shah, MD — June 10, 2025 16:44

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: June 17, 2025 | Acute Follow-Up (DVT)

Encounter	#23 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Acute Follow-Up (DVT)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

One-week follow-up after right calf DVT diagnosed at urgent care.

History of Present Illness:

Right calf pain and swelling improving on apixaban; no bleeding, no chest symptoms. Completed the 7-day loading dose today and transitions to 5 mg BID. Labs 06/10: hemoglobin 13.4, platelets 248, creatinine 0.80, INR 1.0. Provoked distal DVT after travel; no thrombophilia testing indicated. Vascular surgery (Dr. Vasquez) aware; her planned left GSV evaluation will proceed once anticoagulation is complete.

Left leg: still wearing the compression stocking daily; symptoms unchanged.

Vital Signs:

Blood Pressure	134/82 mmHg, seated, left arm
Heart Rate	78 bpm, regular
Temperature	98.2 °F oral
Respiratory Rate	17 /min
SpO2	97% on room air
Weight	171 lb
Height	64 in
BMI	29.3 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of acute right posterior tibial (calf) vein DVT diagnosed 06/10/2025, currently on apixaban. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Acute provoked right posterior tibial vein DVT, treated with apixaban; planned 3-month course through 09/16/2025. Compression stocking for the right leg optional for symptom relief. Repeat right LE duplex at end of therapy.

2. Hypertension, hypothyroidism, hyperlipidemia stable. Return 3 months or sooner.

Orders:

• Apixaban 5 mg BID (continue)

• Right LE venous duplex in 3 months

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Medication	Dose	Frequency	Indication
Apixaban	5 mg	BID	Acute DVT, right calf (3-month course)

Changes since last encounter:

• STOPPED Apixaban 10 mg (06/17/2025)

• STARTED Apixaban 5 mg BID — Acute DVT, right calf (3-month course) (06/17/2025)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
I82.451	Acute DVT right posterior tibial vein
Z79.01	Long-term anticoagulant use

Electronically signed: Dr. Priya Raman, MD — June 17, 2025 14:33

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: July 15, 2025 | Follow-Up (conservative management; interval DVT)

Encounter	#24 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (conservative management; interval DVT)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Left leg symptoms persist on compression; interval right calf DVT.

History of Present Illness:

Interval history: provoked right posterior tibial (calf) DVT on 06/10/2025 after a long car trip, diagnosed at urgent care, on apixaban 5 mg BID through mid-September. Right calf symptoms nearly resolved. The DVT was in the contralateral (right) leg; the left deep system was normal on the 11/2023 duplex and the right-leg duplex of 06/10/2025 noted patent proximal veins.

Left leg: compression 20–30 mmHg worn daily without interruption since 09/17/2024 (10 months), log reviewed. Symptoms persist and are slightly worse during the summer heat even without school: aching and heaviness by mid-afternoon, evening ankle edema, itching medial calf, cramps 1–2 nights per week. Acetaminophen in place of ibuprofen while anticoagulated.

She has arranged respite care for her mother starting October and would like to plan ablation of the left GSV for the fall, after anticoagulation is complete.

Vital Signs:

Blood Pressure	140/79 mmHg, seated, left arm
Heart Rate	77 bpm, regular
Temperature	97.7 °F oral
Respiratory Rate	17 /min
SpO2	98% on room air
Weight	173 lb
Height	64 in
BMI	29.7 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.

Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of acute right posterior tibial (calf) vein DVT diagnosed 06/10/2025, currently on apixaban. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Tortuous varicosities medial calf and distal medial thigh (GSV distribution), unchanged. 1+ pitting ankle edema. Mild xerosis medial calf; no pigmentation, eczema, lipodermatosclerosis or ulcer.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender. Mild residual calf fullness after 06/2025 DVT, no tenderness along the posterior tibial veins.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic left lower extremity superficial venous insufficiency, CEAP C3, persistent after 10 months of uninterrupted compression. Candidate for endovenous ablation of the left GSV; plan after completion of apixaban (09/16/2025) and a normal right-leg follow-up duplex.

2. Right calf DVT, provoked, on treatment — managed by PCP. Not a contraindication to left GSV ablation once anticoagulation is complete and the deep system is confirmed patent on pre-procedure duplex.

3. Continue compression. Return 4 months; order a repeat left venous reflux duplex within 3 months of the planned procedure date.

Orders:

• Continue compression 20–30 mmHg daily

• Return 4 months

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Apixaban	5 mg	BID	Acute DVT, right calf (3-month course)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
I83.812	Varicose veins of left lower extremity with pain
I87.2	Venous insufficiency (chronic) (peripheral)
I82.451	DVT right posterior tibial vein, on treatment

Electronically signed: Dr. Elena Vasquez, MD, FACS — July 15, 2025 15:49

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: August 19, 2025 | Annual Full Skin Examination

Encounter	#25 of 40
Practice	Bayshore Dermatology Group — Dermatology
Location	3100 W Kennedy Blvd, Suite 205 | Tampa, FL 33609 | (813) 555-0177 | NPI: 1639120084
Attending	Dr. Marcus Bell, MD
Encounter Type	Annual Full Skin Examination
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Annual skin check; new spot on right shoulder.

History of Present Illness:

Annual skin exam. New 5 mm pink pearly papule right shoulder, 3 months, bleeds with towel drying. Hands well controlled with emollients. On apixaban for a right calf DVT (06/2025) through mid-September — biopsy bleeding risk discussed; she prefers to proceed with a shave biopsy today with careful hemostasis.

Legs: left varicose veins, wears compression daily; no leg skin complaints.

Vital Signs:

Blood Pressure	138/78 mmHg, seated, left arm
Heart Rate	80 bpm, regular
Temperature	98.2 °F oral
Respiratory Rate	16 /min
SpO2	97% on room air
Weight	171 lb
Height	64 in
BMI	29.3 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of acute right posterior tibial (calf) vein DVT diagnosed 06/10/2025, currently on apixaban. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Skin — Right Shoulder	5 mm pearly pink papule with telangiectasia and a rolled border; dermoscopy with arborizing vessels.
Skin — Trunk/Extremities	Stable seborrheic keratoses. Left medial calf varicosities, no hemosiderin, eczema or ulcer. Compression stocking removed for exam and replaced.
Lymph	No adenopathy.

Assessment & Plan:

1. Suspected basal cell carcinoma, right shoulder: shave biopsy performed under local anesthesia, aluminum chloride hemostasis, pressure dressing; pathology pending. Will call with results and arrange ED&C or excision as appropriate after anticoagulation ends.

2. Seborrheic keratoses, stable. Hand dermatitis, controlled. Left leg varicose veins without stasis skin changes. Return for results / 1 year.

Orders:

• Shave biopsy, right shoulder (11102)

• Dermatopathology

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Apixaban	5 mg	BID	Acute DVT, right calf (3-month course)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Daily broad-spectrum sunscreen SPF 30 or higher on exposed skin; reapply every 2 hours outdoors.

• Monthly self skin examination using the ABCDE rule; photograph any changing lesion and call the office.

• Bland emollient (petrolatum or ceramide cream) to hands twice daily; avoid fragranced soaps.

• Return sooner for any lesion that bleeds, grows, or does not heal within 4 weeks.

Encounter Coding:

Code	Description
99213	Established patient
11102	Tangential biopsy of skin
D48.5	Neoplasm of uncertain behavior of skin

Electronically signed: Dr. Marcus Bell, MD — August 19, 2025 14:54

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: September 16, 2025 | Chronic Care Follow-Up (DVT completion)

Encounter	#26 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Chronic Care Follow-Up (DVT completion)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Completion of 3-month apixaban course; repeat duplex results.

History of Present Illness:

Completes 3 months of apixaban today. Right LE venous duplex 09/12/2025: posterior tibial vein recanalized, no residual thrombus, proximal veins patent. No bleeding complications. Right calf asymptomatic. Shoulder biopsy 08/2025: nodular basal cell carcinoma, ED&C scheduled by dermatology in October now that anticoagulation is finished.

Left leg symptoms persist on daily compression (one year next week); she plans to proceed with vascular surgery's recommendation for ablation this fall.

Vital Signs:

Blood Pressure	125/86 mmHg, seated, left arm
Heart Rate	79 bpm, regular
Temperature	97.7 °F oral
Respiratory Rate	16 /min
SpO2	98% on room air
Weight	173 lb
Height	64 in
BMI	29.7 kg/m²
Pain Score	2/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.

Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Provoked right calf DVT, completed treatment; apixaban discontinued today. Add Z86.718 (personal history of venous thrombosis) to the problem list. No indefinite anticoagulation (provoked, distal, resolved). Travel precautions reviewed.

2. Hypertension, hypothyroidism, hyperlipidemia stable; annual labs next month. Return 1 month for annual physical.

Orders:

• Discontinue apixaban

• Annual fasting labs in 4 weeks

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)

Medication	Dose	Frequency	Indication
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Changes since last encounter:

• STOPPED Apixaban 5 mg (09/16/2025)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
Z86.718	Personal history of other venous thrombosis and embolism

Electronically signed: Dr. Priya Raman, MD — September 16, 2025 15:10

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: October 21, 2025 | Annual Physical / Preventive Visit

Encounter	#27 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Annual Physical / Preventive Visit
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Annual physical.

History of Present Illness:

Annual visit. Labs 10/14/2025 reviewed: CMP normal, LDL 88 on atorvastatin 20 mg, TSH 2.1, A1c 5.6, vitamin D 34. Hypertension at goal. BCC right shoulder treated with ED&C 10/07/2025 by dermatology, healing. Influenza and COVID vaccines today. Mammogram ordered.

Left leg: compression stockings worn daily for over a year (since 09/17/2024); still symptomatic; vascular surgery visit next month to plan ablation.

Vital Signs:

Blood Pressure	142/85 mmHg, seated, left arm
Heart Rate	82 bpm, regular
Temperature	98.2 °F oral
Respiratory Rate	15 /min
SpO2	97% on room air
Weight	171 lb
Height	64 in
BMI	29.3 kg/m²
Pain Score	2/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Preventive visit; vaccines given; mammogram ordered. Chronic conditions stable; continue current medications.

2. History of provoked right calf DVT, resolved. Varicose veins / CVI left leg, symptomatic on compression — per vascular surgery. Return 6 months.

Orders:

• Screening mammogram

• Influenza vaccine; COVID-19 vaccine, administered

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99396	Preventive visit, established
I10	Essential hypertension
E03.9	Hypothyroidism
Z86.718	History of venous thrombosis

Electronically signed: Dr. Priya Raman, MD — October 21, 2025 14:15

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: November 18, 2025 | Follow-Up (pre-procedure planning)

Encounter	#28 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (pre-procedure planning)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Left leg symptoms persist; planning endovenous ablation.

History of Present Illness:

Fourteen months of uninterrupted 20–30 mmHg compression (since 09/17/2024) with elevation, walking and analgesics; log reviewed. Symptoms unchanged to worse: daily afternoon aching and heaviness, evening ankle edema, calf itching, cramps 2 nights per week. Anticoagulation for the right calf DVT completed 09/16/2025; follow-up right duplex 09/12/2025 showed recanalization.

She wished to schedule ablation for December but her mother was hospitalized this month; she asks to defer again to spring 2026. ADL impact continues: stool teaching, excused from standing duties, errands split over two days.

Vital Signs:

Blood Pressure	129/82 mmHg, seated, left arm
Heart Rate	81 bpm, regular
Temperature	97.7 °F oral
Respiratory Rate	15 /min
SpO2	98% on room air
Weight	173 lb
Height	64 in
BMI	29.7 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.

Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Tortuous varicosities medial calf and distal thigh, GSV distribution. 1+ pitting ankle edema at 1:45 pm. Mild xerosis. No pigmentation, eczema or ulcer.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender. Mild residual calf fullness after 06/2025 DVT, no tenderness along the posterior tibial veins.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic left lower extremity superficial venous insufficiency, CEAP C3, failed conservative management > 12 months. Candidate for radiofrequency ablation of the left GSV. Patient defers to spring 2026 for family reasons. Continue compression. Return 4–5 months; repeat left venous reflux duplex to be scheduled within 3 months of the procedure date.

Orders:

• Continue compression 20–30 mmHg daily

• Return 4–5 months

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism

Medication	Dose	Frequency	Indication
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99213	Established patient
I83.812	Varicose veins of left lower extremity with pain
I87.2	Venous insufficiency (chronic) (peripheral)

Electronically signed: Dr. Elena Vasquez, MD, FACS — November 18, 2025 15:31

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: December 9, 2025 | Urgent Care Visit

Encounter	#29 of 40
Practice	Carrollwood Urgent Care — Urgent Care
Location	14030 N Dale Mabry Hwy | Tampa, FL 33618 | (813) 555-0193 | NPI: 1407732915
Attending	Luis Herrera, PA-C
Encounter Type	Urgent Care Visit
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Vomiting and diarrhea x 36 hours.

History of Present Illness:

57-year-old female with 36 hours of nausea, 4 episodes of non-bloody emesis and 6 watery non-bloody stools, low-grade fever, diffuse crampy abdominal pain. Several students with the same illness. Tolerating sips. No recent antibiotics, no travel.

History of right calf DVT 06/2025, anticoagulation completed; no leg symptoms today beyond her chronic left leg aching.

Vital Signs:

Blood Pressure	124/77 mmHg, seated, left arm
Heart Rate	76 bpm, regular
Temperature	98.0 °F oral
Respiratory Rate	16 /min
SpO2	98% on room air
Weight	170 lb
Height	64 in
BMI	29.2 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Mildly dehydrated, non-toxic. HR 92, BP 112/70, T 99.4 °F.
Abdomen	Soft, mild diffuse tenderness, hyperactive bowel sounds, no peritoneal signs.
Extremities	No calf tenderness; left compression stocking in place.

Assessment & Plan:

1. Acute viral gastroenteritis with mild dehydration. Ondansetron ODT 4 mg Q8H PRN x 3 days, oral rehydration, BRAT diet advance as tolerated. Hold losartan for 48 hours while unable to keep fluids down. Return for blood in stool, inability to keep fluids, or fever > 102 °F.

Orders:

• Ondansetron ODT 4 mg #9

• BMP today

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Ondansetron ODT	4 mg	Q8H PRN	Nausea, viral gastroenteritis

Changes since last encounter:

• STARTED Ondansetron ODT 4 mg Q8H PRN — Nausea, viral gastroenteritis (12/09/2025)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Return to urgent care or go to the emergency department for worsening symptoms, high fever, difficulty breathing, chest pain, or new confusion.

• Follow up with your primary care physician within 1–2 weeks. A copy of this visit has been sent to Dr. Priya Raman.

• Rest, fluids, and over-the-counter symptom relief as discussed. Complete the full course of any prescribed antibiotic.

• Work note provided on request.

Encounter Coding:

Code	Description
99213	Urgent care visit
A08.4	Viral intestinal infection, unspecified
E86.0	Dehydration

Electronically signed: Luis Herrera, PA-C — December 9, 2025 9:58

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: January 20, 2026 | Chronic Care Follow-Up (lipids)

Encounter	#30 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Chronic Care Follow-Up (lipids)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Lipid recheck; statin dose.

History of Present Illness:

Fasting lipids 01/13/2026: LDL 112, up from 88 (less walking, holiday diet, admits inconsistent atorvastatin dosing over the holidays). Gastroenteritis in December resolved. BP at goal. Mother's health has stabilized.

Left leg: daily compression continues; still symptomatic; ablation planned for spring with vascular surgery.

Vital Signs:

Blood Pressure	133/78 mmHg, seated, left arm
Heart Rate	66 bpm, regular
Temperature	97.7 °F oral
Respiratory Rate	14 /min
SpO2	98% on room air
Weight	173 lb
Height	64 in
BMI	29.7 kg/m²
Pain Score	4/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Hyperlipidemia, LDL above goal: increase atorvastatin to 40 mg nightly; lipids in 12 weeks; adherence counseling.

2. Hypertension, hypothyroidism stable. History of DVT, resolved. Varicose veins left leg — per vascular surgery. Return 3 months.

Orders:

• Atorvastatin 40 mg nightly, #90

• Lipid panel in 12 weeks

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Changes since last encounter:

• STOPPED Atorvastatin 20 mg (01/20/2026)

• STARTED Atorvastatin 40 mg Nightly — Hyperlipidemia (01/20/2026)

• STOPPED Ondansetron ODT 4 mg (12/12/2025)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99213	Established patient
E78.5	Hyperlipidemia
I10	Essential hypertension

Electronically signed: Dr. Priya Raman, MD — January 20, 2026 15:52

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: February 17, 2026 | Physical Therapy Initial Evaluation

Encounter	#31 of 40
Practice	Tampa Bay Physical Therapy & Rehabilitation — Physical Therapy
Location	4415 W Gandy Blvd | Tampa, FL 33611 | (813) 555-0188 | NPI: 1821456730
Attending	Kevin Nunes, PT, DPT
Encounter Type	Physical Therapy Initial Evaluation
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Low back pain x 3 weeks after lifting a box of textbooks. Referred by Dr. Raman.

History of Present Illness:

58-year-old teacher with 3 weeks of mechanical low back pain after lifting, 5/10 with flexion and prolonged sitting, no radiation below the knee, no numbness, no bowel/bladder change. Lumbar X-ray 02/10/2026: mild L4–L5 and L5–S1 degenerative disc disease, no fracture or spondylolisthesis. Cyclobenzaprine at night from PCP.

Left leg varicose veins with daily compression stocking; left leg aching is chronic and separate from the back complaint.

Vital Signs:

Blood Pressure	139/86 mmHg, seated, left arm
Heart Rate	82 bpm, regular
Temperature	98.1 °F oral
Respiratory Rate	14 /min
SpO2	99% on room air
Weight	175 lb
Height	64 in
BMI	30.0 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Observation	Flattened lumbar lordosis, guarded flexion.
Range of Motion	Lumbar flexion 50% painful, extension 75%, side-bend full.
Neurological	SLR negative bilaterally, reflexes 2+ symmetric, strength 5/5, sensation intact.
Strength / Motor Control	Prone plank 20 s, hip extension 4/5 bilaterally, poor hip-hinge pattern.
Left Lower Extremity	Compression stocking in place; chronic varicosities noted; no neurological findings.

Treatment Log (today's session):

Exercise	Sets x Reps	Load / Band	Tolerance
Prone press-up	3 x 10	—	Centralizes
Dead bug	3 x 10	Body weight	No pain
Hip hinge w/ dowel	3 x 10	Dowel	No pain
Glute bridge	3 x 12	Body weight	1/10

Assessment & Plan:

1. Mechanical low back pain with lumbar degenerative disc disease, flexion-intolerant, with core and hip weakness. Plan 2x/week x 6 weeks: core stabilization, hip hinge training, extension-bias mobility, ergonomic lifting for the classroom.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Cyclobenzaprine	5 mg	QHS PRN	Lumbar muscle spasm

Changes since last encounter:

• STARTED Cyclobenzaprine 5 mg QHS PRN — Lumbar muscle spasm (02/17/2026)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Home exercise program as demonstrated; 1 set of 10–15 repetitions, twice daily, within a pain-free range.

• Ice 15 minutes after exercise if sore. Heat before stretching is acceptable.

• Pain that persists more than 2 hours after exercise means the dose was too high; reduce repetitions and inform the therapist.

• Wear supportive footwear; avoid barefoot walking on hard surfaces.

Encounter Coding:

Code	Description
97161	PT evaluation, low complexity
M54.50	Low back pain, unspecified
M51.36	Other intervertebral disc degeneration, lumbar region

Electronically signed: Kevin Nunes, PT, DPT — February 17, 2026 16:08

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: March 3, 2026 | Physical Therapy Treatment (visit 4 of 12)

Encounter	#32 of 40
Practice	Tampa Bay Physical Therapy & Rehabilitation — Physical Therapy
Location	4415 W Gandy Blvd | Tampa, FL 33611 | (813) 555-0188 | NPI: 1821456730
Attending	Kevin Nunes, PT, DPT
Encounter Type	Physical Therapy Treatment (visit 4 of 12)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 57
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Low back pain, improving.

History of Present Illness:

Pain 2/10 with sitting, 3/10 with lifting. Sleeping better; cyclobenzaprine only twice this week. Home program daily.

Keeps the left compression stocking on; reports the leg still swells by evening.

Vital Signs:

Blood Pressure	142/79 mmHg, seated, left arm
Heart Rate	73 bpm, regular
Temperature	98.3 °F oral
Respiratory Rate	16 /min
SpO2	98% on room air
Weight	176 lb
Height	64 in
BMI	30.2 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.

Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Range of Motion	Lumbar flexion 75%, mildly painful at end range.
Strength	Prone plank 45 s, hip extension 4+/5.

Treatment Log (today's session):

Exercise	Sets x Reps	Load / Band	Tolerance
Bird dog	3 x 10	Body weight	No pain
KB deadlift	3 x 10	15 lb	2/10
Side plank	3 x 20 s	Body weight	No pain
Pallof press	3 x 12	Red band	No pain

Assessment & Plan:

1. Progressing. Advanced core loading and added kettlebell deadlift pattern 15 lb. Continue 2x/week.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Cyclobenzaprine	5 mg	QHS PRN	Lumbar muscle spasm

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Home exercise program as demonstrated; 1 set of 10–15 repetitions, twice daily, within a pain-free range.

• Ice 15 minutes after exercise if sore. Heat before stretching is acceptable.

• Pain that persists more than 2 hours after exercise means the dose was too high; reduce repetitions and inform the therapist.

• Wear supportive footwear; avoid barefoot walking on hard surfaces.

Encounter Coding:

Code	Description
97110	Therapeutic exercise
97140	Manual therapy
M54.50	Low back pain

Electronically signed: Kevin Nunes, PT, DPT — March 3, 2026 12:46

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: March 17, 2026 | Physical Therapy Treatment (visit 8 of 12) — Discharge

Encounter	#33 of 40
Practice	Tampa Bay Physical Therapy & Rehabilitation — Physical Therapy
Location	4415 W Gandy Blvd | Tampa, FL 33611 | (813) 555-0188 | NPI: 1821456730
Attending	Kevin Nunes, PT, DPT
Encounter Type	Physical Therapy Treatment (visit 8 of 12) — Discharge
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 58
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Low back pain, resolved.

History of Present Illness:

Pain 0–1/10; lifting boxes with hinge technique without pain; off cyclobenzaprine. Home program adherence good.

Vital Signs:

Blood Pressure	126/83 mmHg, seated, left arm
Heart Rate	81 bpm, regular
Temperature	97.6 °F oral
Respiratory Rate	14 /min
SpO2	97% on room air
Weight	168 lb
Height	64 in
BMI	28.8 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.

Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Range of Motion	Full, pain-free.
Strength	Prone plank 60 s, hip extension 5/5.

Treatment Log (today's session):

Exercise	Sets x Reps	Load / Band	Tolerance
KB deadlift	3 x 10	25 lb	0/10
Farmer carry	3 x 40 m	20 lb each	0/10
Plank progression	3 x 45 s	Body weight	0/10

Assessment & Plan:

1. Goals met. Discharge to independent home program. Lifting ergonomics handout for classroom. Re-evaluate PRN.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Changes since last encounter:

• STOPPED Cyclobenzaprine 5 mg (03/17/2026)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Home exercise program as demonstrated; 1 set of 10–15 repetitions, twice daily, within a pain-free range.

• Ice 15 minutes after exercise if sore. Heat before stretching is acceptable.

• Pain that persists more than 2 hours after exercise means the dose was too high; reduce repetitions and inform the therapist.

• Wear supportive footwear; avoid barefoot walking on hard surfaces.

Encounter Coding:

Code	Description
97110	Therapeutic exercise

Code	Description
M54.50	Low back pain

Electronically signed: Kevin Nunes, PT, DPT — March 17, 2026 8:24

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: April 14, 2026 | Follow-Up (pre-procedure planning)

Encounter	#34 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (pre-procedure planning)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 58
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Left leg symptoms persist; ready to schedule ablation.

History of Present Illness:

Nineteen months of uninterrupted 20–30 mmHg compression (since 09/17/2024), log reviewed, worn every day including weekends and summer 2025. Symptoms persist and have progressed: aching and heaviness now by late morning on teaching days, ankle and lower calf edema every evening, itching medial calf, cramps 2–3 nights per week. Acetaminophen most days. No ulcer, no bleeding, no phlebitis. Low back pain resolved with PT.

Her mother is now in assisted living and respite is arranged; she would like to schedule the procedure for the summer break.

ADLs: teaches seated for most of the day, no evening walks, split errands, and now reports she had to leave a school field day early because she could not stand for the afternoon session.

Vital Signs:

Blood Pressure	132/80 mmHg, seated, left arm
Heart Rate	80 bpm, regular
Temperature	98.0 °F oral
Respiratory Rate	14 /min
SpO2	98% on room air
Weight	170 lb
Height	64 in
BMI	29.2 kg/m²
Pain Score	2/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.

Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Tortuous varicosities along the distal medial thigh and medial calf in the GSV distribution, more prominent than at prior visits; the largest varicosity at the mid-medial calf now measures 5 mm in diameter on exam with a tape and caliper. Several additional tributary varicosities 3–4 mm. 1+ pitting edema ankle and distal calf at 2:10 pm with stocking worn until arrival. Mild xerosis; no hemosiderin pigmentation, eczema, lipodermatosclerosis or ulceration. Calf soft, non-tender.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender. Mild residual calf fullness after 06/2025 DVT, no tenderness along the posterior tibial veins.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic left lower extremity superficial venous insufficiency with progressing varicosities, CEAP C3 (edema), symptoms unimproved after 19 months of continuous compression plus elevation, exercise and analgesics, with significant interference with work and daily activities.

2. Plan: radiofrequency ablation of the left great saphenous vein (CPT 36475) targeted for summer 2026. Order repeat left lower extremity venous reflux duplex within 3 months of the procedure date (schedule for August). Pre-procedure clearance from PCP. Continue compression. Return after the duplex to finalize and submit prior authorization to Florida Blue.

Orders:

• Lower extremity venous reflux duplex, left, to be performed 08/2026 (93970/93971)

• PCP pre-procedure evaluation

• Continue compression 20–30 mmHg daily

• Return after duplex

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
I83.812	Varicose veins of left lower extremity with pain
I87.2	Venous insufficiency (chronic) (peripheral)

Electronically signed: Dr. Elena Vasquez, MD, FACS — April 14, 2026 9:40

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: May 12, 2026 | Follow-Up (BCC site) / Skin Check

Encounter	#35 of 40
Practice	Bayshore Dermatology Group — Dermatology
Location	3100 W Kennedy Blvd, Suite 205 | Tampa, FL 33609 | (813) 555-0177 | NPI: 1639120084
Attending	Dr. Marcus Bell, MD
Encounter Type	Follow-Up (BCC site) / Skin Check
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 58
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Post-ED&C site check; new spot on nose.

History of Present Illness:

ED&C site right shoulder 10/2025 well healed, no recurrence. New 3 mm pink macule nasal ala, 2 months; dermoscopy without concerning features — monitor. Hands controlled. Wears compression on the left leg daily; no leg skin complaints.

Vital Signs:

Blood Pressure	138/77 mmHg, seated, left arm
Heart Rate	79 bpm, regular
Temperature	98.4 °F oral
Respiratory Rate	14 /min
SpO2	99% on room air
Weight	172 lb
Height	64 in
BMI	29.5 kg/m²
Pain Score	3/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Skin — Right Shoulder	1.2 cm hypopigmented atrophic scar, no nodularity.
Skin — Nose	3 mm pink macule left ala, no arborizing vessels.
Skin — Legs	Left compression stocking in place; distal calf varicosities visible below cuff, no stasis change.

Assessment & Plan:

1. BCC right shoulder, status post ED&C, no recurrence. Nasal ala macule: photograph and recheck 3 months. Return 3 months / annual full exam 08/2026.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Daily broad-spectrum sunscreen SPF 30 or higher on exposed skin; reapply every 2 hours outdoors.

• Monthly self skin examination using the ABCDE rule; photograph any changing lesion and call the office.

• Bland emollient (petrolatum or ceramide cream) to hands twice daily; avoid fragranced soaps.

• Return sooner for any lesion that bleeds, grows, or does not heal within 4 weeks.

Encounter Coding:

Code	Description
99213	Established patient
Z85.828	Personal history of other malignant neoplasm of skin

Electronically signed: Dr. Marcus Bell, MD — May 12, 2026 10:56

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: June 9, 2026 | Chronic Care Follow-Up (lipids)

Encounter	#36 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Chronic Care Follow-Up (lipids)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 58
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Lipid recheck on atorvastatin 40 mg.

History of Present Illness:

LDL 04/2026: 79 on atorvastatin 40 mg with consistent dosing. No myalgias; ALT normal. BP at goal. TSH 2.3.

Left leg: daily compression continues; symptoms persist; vascular surgery is planning ablation with a duplex in August and asked for a pre-procedure evaluation, which we will do in September.

Vital Signs:

Blood Pressure	125/85 mmHg, seated, left arm
Heart Rate	78 bpm, regular
Temperature	97.9 °F oral
Respiratory Rate	14 /min
SpO2	97% on room air
Weight	174 lb
Height	64 in
BMI	29.9 kg/m²
Pain Score	4/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Hyperlipidemia at goal; continue atorvastatin 40 mg. Hypertension, hypothyroidism stable. History of DVT, resolved, no anticoagulation. Pre-procedure evaluation for left GSV ablation to be completed 09/2026. Return 3 months.

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99213	Established patient
E78.5	Hyperlipidemia
I10	Essential hypertension

Electronically signed: Dr. Priya Raman, MD — June 9, 2026 11:12

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: July 7, 2026 | Urgent Care Visit

Encounter	#37 of 40
Practice	Carrollwood Urgent Care — Urgent Care
Location	14030 N Dale Mabry Hwy | Tampa, FL 33618 | (813) 555-0193 | NPI: 1407732915
Attending	Dr. Omar Siddiqui, DO
Encounter Type	Urgent Care Visit
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 58
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Right ankle injury stepping off a curb this morning.

History of Present Illness:

58-year-old female inverted the right ankle stepping off a curb 4 hours ago; immediate lateral pain and swelling, able to bear weight for 4 steps with pain. No prior ankle injury. Ottawa ankle rules: tenderness at the lateral malleolus tip — X-ray obtained.

History of right calf DVT 06/2025, treated; left leg varicose veins, compression stocking worn daily.

Vital Signs:

Blood Pressure	131/82 mmHg, seated, left arm
Heart Rate	77 bpm, regular
Temperature	98.3 °F oral
Respiratory Rate	14 /min
SpO2	98% on room air
Weight	176 lb
Height	64 in
BMI	30.2 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

Right Ankle	Moderate lateral swelling and ecchymosis; tenderness ATFL and lateral malleolus tip; anterior drawer mild laxity with firm end point; talar tilt negative; neurovascularly intact; squeeze test negative.
Right Calf	No calf tenderness or swelling.
Left Lower Extremity	Compression stocking in place; chronic varicosities.

Assessment & Plan:

1. Right lateral ankle sprain, grade II (ATFL); X-ray negative for fracture (see report). RICE, lace-up ankle brace, naproxen 500 mg BID x 7 days with food, early range of motion. Return for inability to bear weight, numbness or worsening swelling. Follow up PCP if not improving in 2 weeks.

Orders:

• X-ray right ankle, 3 views (73610)

• Naproxen 500 mg #14

• Lace-up ankle brace

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency
Naproxen	500 mg	BID x 7 days	Right ankle sprain

Changes since last encounter:

• STARTED Naproxen 500 mg BID x 7 days — Right ankle sprain (07/07/2026)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Return to urgent care or go to the emergency department for worsening symptoms, high fever, difficulty breathing, chest pain, or new confusion.

• Follow up with your primary care physician within 1–2 weeks. A copy of this visit has been sent to Dr. Priya Raman.

• Rest, fluids, and over-the-counter symptom relief as discussed. Complete the full course of any prescribed antibiotic.

• Work note provided on request.

Encounter Coding:

Code	Description
99213	Urgent care visit
73610	X-ray ankle 3 views
S93.401A	Sprain of unspecified ligament of right ankle, initial

Electronically signed: Dr. Omar Siddiqui, DO — July 7, 2026 12:28

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: August 11, 2026 | Follow-Up (procedure scheduling)

Encounter	#38 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (procedure scheduling)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 58
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Scheduling left GSV ablation; duplex pending.

History of Present Illness:

Twenty-three months of uninterrupted compression (since 09/17/2024). Symptoms unchanged from April: aching and heaviness by late morning, evening edema, itching, cramps 2–3 nights per week; she was unable to stand through a 3-hour school registration event last week and had to sit at a side table. Ankle sprain last month resolved. Procedure date set for 10/2026 pending prior authorization; duplex scheduled 08/25/2026.

Vital Signs:

Blood Pressure	129/81 mmHg, seated, left arm
Heart Rate	80 bpm, regular
Temperature	97.9 °F oral
Respiratory Rate	17 /min
SpO2	97% on room air
Weight	174 lb
Height	64 in
BMI	29.9 kg/m²
Pain Score	5/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.

Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Tortuous varicosities distal medial thigh and medial calf in the GSV distribution, unchanged from April. 1+ pitting edema ankle and distal calf. No pigmentation, eczema or ulcer.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender. Mild residual calf fullness after 06/2025 DVT, no tenderness along the posterior tibial veins.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic left lower extremity superficial venous insufficiency, CEAP C3, failed conservative management. Proceed with pre-procedure duplex 08/25/2026 and PCP clearance 09/2026; final visit 09/22/2026 to review and submit prior authorization.

Orders:

• Lower extremity venous reflux duplex, left, 08/25/2026

• Return 09/22/2026

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)

Medication	Dose	Frequency	Indication
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Changes since last encounter:

• STOPPED Naproxen 500 mg (07/14/2026)

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99213	Established patient
I83.812	Varicose veins of left lower extremity with pain
I87.2	Venous insufficiency (chronic) (peripheral)

Electronically signed: Dr. Elena Vasquez, MD, FACS — August 11, 2026 11:33

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: September 8, 2026 | Pre-Procedure Evaluation

Encounter	#39 of 40
Practice	Westchase Family Medicine — Primary Care
Location	11970 Sheldon Rd, Suite 120 | Tampa, FL 33626 | (813) 555-0161 | NPI: 1750233419
Attending	Dr. Priya Raman, MD
Encounter Type	Pre-Procedure Evaluation
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 58
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Pre-procedure evaluation for left great saphenous vein radiofrequency ablation.

History of Present Illness:

Requested by Dr. Vasquez for office-based radiofrequency ablation of the left GSV under tumescent local anesthesia, planned 10/2026. Hypertension at goal, hypothyroidism euthyroid, hyperlipidemia at goal. Provoked right calf DVT 06/2025, treated and resolved, off anticoagulation since 09/16/2025 with a normal follow-up duplex. No bleeding disorder. Functional capacity > 4 METs (climbs two flights). No anesthesia history problems. Labs 09/01/2026: CBC and BMP normal.

Left leg symptoms continue daily despite two years of compression; she reports she now sits for most of the teaching day and needs help with grocery shopping.

Vital Signs:

Blood Pressure	135/78 mmHg, seated, left arm
Heart Rate	79 bpm, regular
Temperature	98.3 °F oral
Respiratory Rate	17 /min
SpO2	98% on room air
Weight	176 lb
Height	64 in
BMI	30.2 kg/m²
Pain Score	6/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.

Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Abdomen	Soft, non-tender, non-distended, no organomegaly.
Skin	Warm and dry. No rash unless noted. No open wounds.
Neurological	Cranial nerves II–XII grossly intact. Strength 5/5 throughout. Gait normal.
Psychiatric	Normal mood and affect, appropriate judgment and insight.

Assessment & Plan:

1. Low-risk office procedure in a patient with well-controlled chronic conditions: acceptable risk, no further testing needed. Continue all medications. History of provoked DVT noted; post-procedure ambulation and compression per vascular surgery; no peri-procedural anticoagulation indicated. Copy of this note to Gulf Coast Vascular Surgery Associates.

Orders:

• Clearance letter faxed to vascular surgery

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Take medications as prescribed. Bring all medication bottles to each visit for reconciliation.

• Home blood pressure log: measure seated, after 5 minutes of rest, morning and evening, 3 days per week. Bring the log.

• DASH-style diet, sodium under 2 g/day, limit alcohol to 1 drink/day or less.

• Fasting labs as ordered; nothing to eat or drink except water for 10–12 hours before the draw.

• Annual preventive services reviewed; see the health maintenance table in your patient portal.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
Z01.818	Encounter for other preprocedural examination

Electronically signed: Dr. Priya Raman, MD — September 8, 2026 12:49

Copy to: chart; released to Gulf Coast Vascular Surgery Associates for prior authorization.

Visit Date: September 22, 2026 | Follow-Up (duplex review; prior authorization)

Encounter	#40 of 40
Practice	Gulf Coast Vascular Surgery Associates — Vascular Surgery
Location	2200 Bayshore Blvd, Suite 310 | Tampa, FL 33606 | (813) 555-0140 | NPI: 1987654321
Attending	Dr. Elena Vasquez, MD, FACS
Encounter Type	Follow-Up (duplex review; prior authorization)
Patient	Elena Ruiz | MRN GCV-012877 | DOB March 4, 1968 | Age 58
Primary Care	Dr. Priya Raman, MD
Payer	Florida Blue PPO — BlueOptions, member FLB771204936

Chief Complaint:

Review of 08/25/2026 left venous reflux duplex; persistent symptoms; prior authorization for RFA.

History of Present Illness:

Returns to review the left lower extremity venous reflux duplex of 08/25/2026: great saphenous vein incompetent with saphenofemoral junction reflux of 2.1 seconds and axial reflux through the thigh and calf; GSV 7.2 mm at the SFJ and 5.8 mm at mid-thigh; deep system patent with no DVT or deep reflux; small saphenous competent; no incompetent perforators. PCP pre-procedure evaluation 09/08/2026 reviewed: cleared.

Compression history for the prior authorization record: 20–30 mmHg graduated knee-high stockings first started 02/13/2024 and interrupted at the beginning of May 2024 (patient stopped for the summer); restarted 09/17/2024 and worn daily without interruption since 09/17/2024 (24 months as of today), documented at every visit, with elevation, walking and analgesics throughout.

Symptoms today: aching and heaviness of the left leg every day beginning by late morning, visible swelling of the ankle and lower calf every evening, itching over the medial calf, night cramps 2–3 times per week. Acetaminophen most days. No improvement at any point during the 24 months of continuous compression; by her account the leg is worse than when the stockings were restarted.

Activities of daily living: she cannot stand through a full teaching day and now teaches seated for most of it; she has given up evening walks and her Saturday volunteer shift at the farmers market; grocery shopping is done by her daughter because she cannot stand through the checkout line; she had to leave a school field day and a registration event early this year.

No ulcer, no bleeding from a varicosity, no superficial thrombophlebitis. Contralateral provoked calf DVT 06/2025, treated and resolved; the left deep system is normal on the 08/25/2026 duplex.

Vital Signs:

Blood Pressure	138/82 mmHg, seated, left arm
Heart Rate	70 bpm, regular
Temperature	97.6 °F oral
Respiratory Rate	15 /min
SpO2	97% on room air
Weight	168 lb
Height	64 in
BMI	28.8 kg/m²
Pain Score	4/10

Review of Systems:

Constitutional	Denies fever, chills, unintentional weight loss, night sweats. Energy at baseline.
Eyes	Denies visual change, eye pain, discharge.
ENT	Denies hearing loss, tinnitus, sore throat, sinus pressure unless noted in HPI.
Cardiovascular	Denies chest pain, palpitations, orthopnea, PND, syncope.
Respiratory	Denies dyspnea at rest, wheeze, hemoptysis. Cough only if noted in HPI.
Gastrointestinal	Denies abdominal pain, nausea, vomiting, melena, hematochezia, change in bowel habit unless noted in HPI.
Genitourinary	Denies dysuria, hematuria, frequency.
Musculoskeletal	Joint complaints as noted in HPI; otherwise denies joint swelling, morning stiffness.
Extremities / Vascular	Chronic left lower extremity aching, heaviness and evening swelling, followed by vascular surgery. No claudication. No rest pain. No ulceration or bleeding from veins.
Skin	Denies new rashes, non-healing wounds or ulcers unless noted in HPI.
Neurological	Denies headache, focal weakness, numbness, dizziness.
Psychiatric	Denies depressed mood, anxiety beyond situational stress. Sleep adequate except when leg cramps wake her.
Endocrine	Hypothyroidism on replacement; denies heat/cold intolerance, polyuria, polydipsia.
Hematologic / Lymphatic	History of right calf (posterior tibial vein) DVT 06/2025, treated with a 3-month course of apixaban, completed 09/16/2025. Denies easy bruising or bleeding.
Allergic / Immunologic	Seasonal rhinitis in spring; no anaphylaxis history.

Physical Examination:

General	Well-appearing woman in no acute distress, alert, cooperative, ambulating independently.
HEENT	Normocephalic, atraumatic. Pupils equal and reactive. Oropharynx clear. Neck supple, no thyromegaly.
Cardiovascular	Regular rate and rhythm, no murmurs, rubs or gallops. Radial pulses 2+ bilaterally.
Respiratory	Clear to auscultation bilaterally, no wheezes, rales or rhonchi. Normal effort.
Left Lower Extremity	Tortuous varicosities distal medial thigh and medial calf in the GSV distribution, largest 5 mm (unchanged from 04/14/2026). 1+ pitting edema of the ankle and distal calf at 11:30 am with stocking worn until arrival. Mild xerosis medial calf; no hemosiderin pigmentation, eczema, lipodermatosclerosis or ulceration. Calf soft, non-tender; no cords.
Right Lower Extremity	Scattered reticular veins lateral thigh. No varicosities, edema, pigmentation or ulceration. Calf soft, non-tender. Mild residual calf fullness after 06/2025 DVT, no tenderness along the posterior tibial veins.
Pulses	Femoral, popliteal, dorsalis pedis and posterior tibial pulses 2+ and symmetric bilaterally. Capillary refill < 2 seconds.
Neurological	Sensation intact to light touch both feet. Motor 5/5.

Assessment & Plan:

1. Symptomatic chronic venous insufficiency of the left lower extremity due to great saphenous vein reflux (SFJ reflux 2.1 s, GSV 7.2 mm at SFJ) with varicosities up to 5 mm, CEAP clinical class C3 (varicose veins with edema), Ep, As, Pr.

2. Persistent pain, swelling, itching and cramping attributed to saphenous reflux, significantly interfering with work and daily activities, after 24 months of continuous 20–30 mmHg compression (09/17/2024 to 09/22/2026) plus elevation, exercise and analgesics without improvement.

3. Recommend endovenous radiofrequency ablation of the left great saphenous vein (CPT 36475). Risks, benefits and alternatives reviewed; patient wishes to proceed. Submit prior authorization to Florida Blue today. Continue compression until the procedure.

Orders:

• Prior authorization: CPT 36475, left GSV, Florida Blue PPO

• RFA left GSV, office procedure suite, 10/2026 pending PA

Medication Reconciliation (active at this encounter):

Medication	Dose	Frequency	Indication
Losartan	100 mg	Daily	Hypertension
Levothyroxine	75 mcg	Daily	Hypothyroidism
Acetaminophen	500 mg	PRN	Left leg aching (NSAIDs held with anticoagulation; continued by preference)
Atorvastatin	40 mg	Nightly	Hyperlipidemia
Cholecalciferol (Vitamin D3)	2000 IU	Daily	Vitamin D insufficiency

Allergies: Cephalexin — rash (08/2024, mild, non-urticarial).

Patient Instructions / After Visit Summary:

• Elevate the legs above heart level for 15–20 minutes, 3–4 times daily, and at the end of the work day.

• Avoid prolonged standing in one place; shift weight, walk briefly, and flex the calves when standing is unavoidable.

• Walk 30 minutes daily; the calf muscle pump is the most effective natural treatment for venous congestion.

• Put compression stockings on first thing in the morning before swelling develops; remove at bedtime. Replace every 4–6 months as elastic wears.

• Call the office for sudden one-sided calf swelling, redness or warmth, chest pain or shortness of breath, or any bleeding from a vein.

• Maintain a healthy weight; each additional 10 lb increases venous pressure at the ankle.

Encounter Coding:

Code	Description
99214	Established patient, moderate complexity
I83.812	Varicose veins of left lower extremity with pain
I87.2	Venous insufficiency (chronic) (peripheral)

Electronically signed: Dr. Elena Vasquez, MD, FACS — September 22, 2026 8:27

Copy to: Dr. Priya Raman, MD; released to Gulf Coast Vascular Surgery Associates for prior authorization.

8. DIAGNOSTIC IMAGING & TESTING REPORTS

Lower Extremity Venous Reflux Duplex Ultrasound — Left — November 14, 2023

Performed At	Gulf Coast Vascular Surgery Associates / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Physician	Dr. Elena Vasquez, MD, FACS
Indication	Left leg heaviness, aching, evening edema; visible medial calf veins; evaluate for reflux
Technique	Standing and reverse Trendelenburg; compression, color and spectral Doppler; Valsalva and distal augmentation/release; 7–12 MHz linear transducer
Deep System	Common femoral, femoral, profunda, popliteal, posterior tibial and peroneal veins compressible with spontaneous, phasic, augmentable flow. No acute or chronic thrombus. No deep venous reflux.
Saphenofemoral Junction	Brief retrograde flow 0.4 seconds with Valsalva (below the 0.5 second threshold for pathologic reflux).
Great Saphenous Vein	Diameter 3.9 mm at the SFJ, 3.2 mm mid-thigh, 2.8 mm at the knee. No sustained reflux in the thigh or calf segments (retrograde flow < 0.5 seconds).
Small Saphenous Vein	Competent. Diameter 2.4 mm. No reflux.
Perforators	No incompetent perforators identified.
Tributaries	Small reticular veins and tributaries medial calf, 1–2 mm.

Interpretation: No deep venous thrombosis. Borderline, sub-threshold saphenofemoral junction reflux (0.4 s) with a non-dilated great saphenous vein. No pathologic superficial or deep reflux by this laboratory's criteria (≥ 0.5 s). Correlate clinically; repeat if symptoms progress.

Electronically signed: Dr. Thomas Reed, RVT / Dr. Elena Vasquez, MD — November 14, 2023

Screening Mammogram, Bilateral — November 21, 2023

Performed At	Westchase Family Medicine / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Physician	Dr. Priya Raman, MD
Technique	Digital breast tomosynthesis, CC and MLO views bilaterally
Breast Density	Scattered fibroglandular densities (B)
Findings	No masses, suspicious calcifications or architectural distortion. Stable compared with 2022.
Assessment	BI-RADS 1 — Negative

Interpretation: Negative screening mammogram. Routine annual screening recommended.

Electronically signed: Dr. Naomi Feld, MD (Radiology) — November 21, 2023

X-Ray Right Knee, 3 Views — March 12, 2024

Performed At	Westchase Family Medicine / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Physician	Dr. Priya Raman, MD

Indication	Anterior knee pain x 6 weeks
Findings	No fracture or dislocation. Mild patellofemoral joint space narrowing with small osteophyte at the superior pole of the patella. Tibiofemoral compartments preserved. No effusion.
Soft Tissues	Unremarkable.

Interpretation: Mild patellofemoral degenerative change. No acute abnormality.

Electronically signed: Dr. Naomi Feld, MD (Radiology) — March 12, 2024

Screening Mammogram, Bilateral — November 19, 2024

Performed At	Westchase Family Medicine / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Physician	Dr. Priya Raman, MD
Technique	Digital breast tomosynthesis
Findings	Stable 6 mm circumscribed oval mass left breast upper outer quadrant, present since 2022, consistent with a benign cyst/fibroadenoma. No suspicious findings.
Assessment	BI-RADS 2 — Benign

Interpretation: Benign findings. Routine annual screening.

Electronically signed: Dr. Naomi Feld, MD (Radiology) — November 19, 2024

X-Ray Chest, PA and Lateral — January 14, 2025

Performed At	Carrollwood Urgent Care / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Provider	Dr. Omar Siddiqui, DO
Indication	Cough x 9 days, chest tightness
Findings	Lungs clear without consolidation, effusion or pneumothorax. Heart size normal. Mediastinal contours normal. Bony thorax intact.

Interpretation: No acute cardiopulmonary process.

Electronically signed: Dr. Naomi Feld, MD (Radiology) — January 14, 2025

Venous Duplex Ultrasound, Right Lower Extremity — June 10, 2025

Performed At	Carrollwood Urgent Care / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Provider	Dr. Anita Shah, MD
Indication	Right calf pain and swelling after prolonged travel; rule out DVT
Common Femoral / Femoral / Profunda	Fully compressible, normal phasic flow, no thrombus.
Popliteal Vein	Compressible, no thrombus.

Posterior Tibial Veins	One of the paired posterior tibial veins is non-compressible with echogenic intraluminal material over a 6 cm segment in the proximal-to-mid calf; non-occlusive with partial flow around the thrombus. Findings consistent with acute thrombus.
Peroneal Veins	Compressible, no thrombus.
Gastrocnemius / Soleal Veins	Compressible.
Great Saphenous Vein (right)	Compressible, no superficial thrombophlebitis.

Interpretation: Acute non-occlusive deep vein thrombosis of a right posterior tibial vein (isolated distal/calf DVT). No proximal DVT. Clinical correlation and treatment per ordering provider.

Electronically signed: Carrollwood Imaging — Dr. Sanjay Mehta, MD (Radiology) — June 10, 2025

Venous Duplex Ultrasound, Right Lower Extremity — September 12, 2025

Performed At	Westchase Family Medicine / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Physician	Dr. Priya Raman, MD
Indication	Follow-up right posterior tibial vein DVT (06/10/2025) at completion of anticoagulation
Deep System	Common femoral, femoral, profunda and popliteal veins compressible without thrombus.
Posterior Tibial Veins	Previously thrombosed posterior tibial vein now fully compressible with restored flow; no residual thrombus; minimal wall thickening.
Peroneal Veins	Compressible.

Interpretation: Interval complete recanalization of the right posterior tibial vein DVT. No residual or new thrombus.

Electronically signed: Carrollwood Imaging — Dr. Sanjay Mehta, MD (Radiology) — September 12, 2025

Screening Mammogram, Bilateral — November 4, 2025

Performed At	Westchase Family Medicine / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Physician	Dr. Priya Raman, MD
Findings	Stable benign left breast mass. No suspicious findings.
Assessment	BI-RADS 2 — Benign

Interpretation: Benign. Routine annual screening.

Electronically signed: Dr. Naomi Feld, MD (Radiology) — November 4, 2025

X-Ray Lumbar Spine, 2 Views — February 10, 2026

Performed At	Westchase Family Medicine / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Physician	Dr. Priya Raman, MD

Indication	Low back pain after lifting
Findings	Mild disc space narrowing at L4–L5 and L5–S1 with small anterior osteophytes. No fracture, spondylolisthesis or pars defect. Sacroiliac joints normal.

Interpretation: Mild lumbar degenerative disc disease. No acute abnormality.

Electronically signed: Dr. Naomi Feld, MD (Radiology) — February 10, 2026

X-Ray Right Ankle, 3 Views — July 7, 2026

Performed At	Carrollwood Urgent Care / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Provider	Dr. Omar Siddiqui, DO
Indication	Inversion injury, lateral malleolus tenderness
Findings	No fracture or dislocation. Ankle mortise congruent. Lateral soft tissue swelling. No os trigonum or avulsion fragment.

Interpretation: No fracture. Lateral soft tissue swelling consistent with sprain.

Electronically signed: Dr. Naomi Feld, MD (Radiology) — July 7, 2026

Lower Extremity Venous Reflux Duplex Ultrasound — Left — August 25, 2026

Performed At	Gulf Coast Vascular Surgery Associates / affiliated imaging
Patient	Elena Ruiz | MRN GCV-012877
Ordering Physician	Dr. Elena Vasquez, MD, FACS
Indication	Symptomatic varicose veins and chronic venous insufficiency, left leg, failed conservative management; pre-procedure mapping for GSV ablation
Technique	Standing; compression, color and spectral Doppler; Valsalva and distal augmentation/release; 7–12 MHz linear transducer; GSV mapped from SFJ to ankle
Deep System	Common femoral, femoral, profunda, popliteal, posterior tibial and peroneal veins compressible with normal phasic, augmentable flow. No acute or chronic thrombus. No deep venous reflux.
Saphenofemoral Junction	Incompetent. Reflux 2.1 seconds with Valsalva.
Great Saphenous Vein	Incompetent with axial reflux. Diameter 7.2 mm at the SFJ, 5.8 mm mid-thigh, 4.6 mm at the knee, 3.9 mm mid-calf. Reflux 1.4–1.9 seconds in the thigh segments and 1.1 seconds at the knee on augmentation release. Straight course, suitable for catheter access at the knee. Depth from skin 1.1 cm at mid-thigh.
Small Saphenous Vein	Competent. Diameter 2.6 mm. No reflux.
Perforators	No incompetent perforators (none ≥ 3.5 mm with outward flow > 0.5 s).
Tributaries	Dilated tortuous tributary varicosities medial calf arising from the GSV, 3–5 mm, largest 5 mm.

Interpretation: Left great saphenous vein incompetence with pathologic saphenofemoral junction reflux (2.1 s) and axial reflux through the thigh and calf; GSV 7.2 mm at the SFJ. No deep venous thrombosis and no deep venous reflux. Small saphenous and perforators competent. Findings consistent with symptomatic superficial venous

insufficiency; anatomy suitable for endovenous thermal ablation. Interval progression compared with 11/14/2023 (previously 0.4 s, 3.9 mm).

Electronically signed: Dr. Thomas Reed, RVT / Dr. Elena Vasquez, MD — August 25, 2026

9. LABORATORY RESULTS

Laboratory Report — October 10, 2023

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	October 10, 2023 07:13 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Complete Blood Count

Test	Result	Reference Range	Flag
WBC	6.1	4.0–10.5 K/uL	
Hemoglobin	13.6	11.5–15.5 g/dL	
Hematocrit	40.8	35–45 %	
MCV	89	80–100 fL	
Platelets	262	150–400 K/uL	

Comprehensive Metabolic Panel

Test	Result	Reference Range	Flag
Sodium	139	135–145 mmol/L	
Potassium	4.1	3.5–5.1 mmol/L	
Chloride	103	98–107 mmol/L	
CO2	25	22–29 mmol/L	
BUN	14	7–20 mg/dL	
Creatinine	0.78	0.5–1.0 mg/dL	
eGFR	> 90	> 60 mL/min/1.73m²	
Glucose, fasting	92	70–99 mg/dL	
Calcium	9.4	8.6–10.2 mg/dL	
Total Protein	7.1	6.4–8.3 g/dL	
Albumin	4.3	3.5–5.0 g/dL	
AST	22	10–35 U/L	
ALT	19	7–35 U/L	
Alk Phos	68	38–126 U/L	
Total Bilirubin	0.6	0.2–1.2 mg/dL	

Lipid Panel

Test	Result	Reference Range	Flag
Total Cholesterol	236	< 200 mg/dL	H
LDL Cholesterol (calc)	158	< 100 mg/dL	H
HDL Cholesterol	51	> 50 mg/dL	
Triglycerides	135	< 150 mg/dL	

Test	Result	Reference Range	Flag
Non-HDL	185	< 130 mg/dL	H

Thyroid

Test	Result	Reference Range	Flag
TSH	3.1	0.40–4.50 mIU/L	

Comment: LDL above goal; flagged for follow-up.

Reviewed: Dr. Priya Raman, MD — October 11, 2023

Laboratory Report — February 6, 2024

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	February 6, 2024 07:32 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Basic Metabolic Panel

Test	Result	Reference Range	Flag
Sodium	138	135–145 mmol/L	
Potassium	4.3	3.5–5.1 mmol/L	
Chloride	102	98–107 mmol/L	
CO2	24	22–29 mmol/L	
BUN	15	7–20 mg/dL	
Creatinine	0.82	0.5–1.0 mg/dL	
Glucose	94	70–99 mg/dL	
Calcium	9.3	8.6–10.2 mg/dL	

Comment: Post losartan increase; potassium and creatinine stable.

Reviewed: Dr. Priya Raman, MD — February 7, 2024

Laboratory Report — May 14, 2024

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	May 14, 2024 07:30 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Lipid Panel

Test	Result	Reference Range	Flag
Total Cholesterol	238	< 200 mg/dL	H
LDL Cholesterol (calc)	161	< 100 mg/dL	H
HDL Cholesterol	52	> 50 mg/dL	

Test	Result	Reference Range	Flag
Triglycerides	128	< 150 mg/dL	
Non-HDL	186	< 130 mg/dL	H

Comprehensive Metabolic Panel

Test	Result	Reference Range	Flag
Sodium	139	135–145 mmol/L	
Potassium	4.2	3.5–5.1 mmol/L	
Chloride	103	98–107 mmol/L	
CO2	25	22–29 mmol/L	
BUN	14	7–20 mg/dL	
Creatinine	0.8	0.5–1.0 mg/dL	
eGFR	> 90	> 60 mL/min/1.73m²	
Glucose, fasting	95	70–99 mg/dL	
Calcium	9.4	8.6–10.2 mg/dL	
Total Protein	7.1	6.4–8.3 g/dL	
Albumin	4.3	3.5–5.0 g/dL	
AST	22	10–35 U/L	
ALT	21	7–35 U/L	
Alk Phos	68	38–126 U/L	
Total Bilirubin	0.6	0.2–1.2 mg/dL	

Comment: Fasting. LDL 161 despite lifestyle change.

Reviewed: Dr. Priya Raman, MD — May 15, 2024

Laboratory Report — August 20, 2024

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	August 20, 2024 07:28 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Lipid Panel

Test	Result	Reference Range	Flag
Total Cholesterol	171	< 200 mg/dL	
LDL Cholesterol (calc)	96	< 100 mg/dL	
HDL Cholesterol	54	> 50 mg/dL	
Triglycerides	105	< 150 mg/dL	
Non-HDL	117	< 130 mg/dL	

Hepatic Function

Test	Result	Reference Range	Flag
ALT	24	7–35 U/L	
AST	21	10–35 U/L	

Comment: On atorvastatin 20 mg x 12 weeks.

Reviewed: Dr. Priya Raman, MD — August 21, 2024

Laboratory Report — September 24, 2024

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	September 24, 2024 07:13 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Thyroid

Test	Result	Reference Range	Flag
TSH	5.9	0.40–4.50 mIU/L	H
Free T4	0.9	0.8–1.8 ng/dL	

Vitamin D

Test	Result	Reference Range	Flag
25-OH Vitamin D	22	30–100 ng/mL	L

Complete Blood Count

Test	Result	Reference Range	Flag
WBC	5.8	4.0–10.5 K/uL	
Hemoglobin	13.2	11.5–15.5 g/dL	
Hematocrit	39.6	35–45 %	
MCV	89	80–100 fL	
Platelets	255	150–400 K/uL	

Comment: TSH elevated; vitamin D insufficient.

Reviewed: Dr. Priya Raman, MD — September 25, 2024

Laboratory Report — December 10, 2024

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	December 10, 2024 07:40 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Thyroid

Test	Result	Reference Range	Flag
TSH	2.4	0.40–4.50 mIU/L	

Comment: After levothyroxine increase to 75 mcg.

Reviewed: Dr. Priya Raman, MD — December 11, 2024

Laboratory Report — June 10, 2025

Ordering Provider	Dr. Anita Shah, MD
Practice	Carrollwood Urgent Care
Collected	June 10, 2025 07:22 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Complete Blood Count

Test	Result	Reference Range	Flag
WBC	7.0	4.0–10.5 K/uL	
Hemoglobin	13.4	11.5–15.5 g/dL	
Hematocrit	40.2	35–45 %	
MCV	89	80–100 fL	
Platelets	248	150–400 K/uL	

Basic Metabolic Panel

Test	Result	Reference Range	Flag
Sodium	138	135–145 mmol/L	
Potassium	4.2	3.5–5.1 mmol/L	
Chloride	102	98–107 mmol/L	
CO2	24	22–29 mmol/L	
BUN	15	7–20 mg/dL	
Creatinine	0.8	0.5–1.0 mg/dL	
Glucose	94	70–99 mg/dL	
Calcium	9.3	8.6–10.2 mg/dL	

Coagulation

Test	Result	Reference Range	Flag
PT	11.8	11.0–13.5 s	
INR	1.0	0.8–1.2	
aPTT	29	25–35 s	

Comment: Baseline prior to apixaban.

Reviewed: Dr. Anita Shah, MD — June 11, 2025

Laboratory Report — October 14, 2025

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	October 14, 2025 07:48 (fasting)

Performing Lab Tampa Bay Clinical Laboratories, CLIA 10D0999999

Patient Elena Ruiz | MRN GCV-012877

Complete Blood Count

Test	Result	Reference Range	Flag
WBC	5.9	4.0–10.5 K/uL	
Hemoglobin	13.5	11.5–15.5 g/dL	
Hematocrit	40.5	35–45 %	
MCV	89	80–100 fL	
Platelets	251	150–400 K/uL	

Comprehensive Metabolic Panel

Test	Result	Reference Range	Flag
Sodium	139	135–145 mmol/L	
Potassium	4.2	3.5–5.1 mmol/L	
Chloride	103	98–107 mmol/L	
CO2	25	22–29 mmol/L	
BUN	14	7–20 mg/dL	
Creatinine	0.81	0.5–1.0 mg/dL	
eGFR	> 90	> 60 mL/min/1.73m²	
Glucose, fasting	96	70–99 mg/dL	
Calcium	9.4	8.6–10.2 mg/dL	
Total Protein	7.1	6.4–8.3 g/dL	
Albumin	4.3	3.5–5.0 g/dL	
AST	22	10–35 U/L	
ALT	23	7–35 U/L	
Alk Phos	68	38–126 U/L	
Total Bilirubin	0.6	0.2–1.2 mg/dL	

Lipid Panel

Test	Result	Reference Range	Flag
Total Cholesterol	160	< 200 mg/dL	
LDL Cholesterol (calc)	88	< 100 mg/dL	
HDL Cholesterol	55	> 50 mg/dL	
Triglycerides	85	< 150 mg/dL	
Non-HDL	105	< 130 mg/dL	

Thyroid

Test	Result	Reference Range	Flag
TSH	2.1	0.40–4.50 mIU/L	

Hemoglobin A1c

Test	Result	Reference Range	Flag
HbA1c	5.6	< 5.7 %	

Vitamin D

Test	Result	Reference Range	Flag
25-OH Vitamin D	34	30–100 ng/mL	

Comment: Annual labs.

Reviewed: Dr. Priya Raman, MD — October 15, 2025

Laboratory Report — December 9, 2025

Ordering Provider	Luis Herrera, PA-C
Practice	Carrollwood Urgent Care
Collected	December 9, 2025 07:54 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Basic Metabolic Panel

Test	Result	Reference Range	Flag
Sodium	138	135–145 mmol/L	
Potassium	3.6	3.5–5.1 mmol/L	
Chloride	102	98–107 mmol/L	
CO2	24	22–29 mmol/L	
BUN	15	7–20 mg/dL	
Creatinine	0.92	0.5–1.0 mg/dL	
Glucose	94	70–99 mg/dL	
Calcium	9.3	8.6–10.2 mg/dL	

Comment: Mild dehydration; potassium low-normal.

Reviewed: Luis Herrera, PA-C — December 10, 2025

Laboratory Report — January 13, 2026

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	January 13, 2026 07:39 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Lipid Panel

Test	Result	Reference Range	Flag
Total Cholesterol	188	< 200 mg/dL	
LDL Cholesterol (calc)	112	< 100 mg/dL	H
HDL Cholesterol	53	> 50 mg/dL	

Test	Result	Reference Range	Flag
Triglycerides	115	< 150 mg/dL	
Non-HDL	135	< 130 mg/dL	H

Comment: LDL up; adherence discussed.

Reviewed: Dr. Priya Raman, MD — January 14, 2026

Laboratory Report — April 21, 2026

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	April 21, 2026 07:37 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Lipid Panel

Test	Result	Reference Range	Flag
Total Cholesterol	151	< 200 mg/dL	
LDL Cholesterol (calc)	79	< 100 mg/dL	
HDL Cholesterol	56	> 50 mg/dL	
Triglycerides	80	< 150 mg/dL	
Non-HDL	95	< 130 mg/dL	

Hepatic Function

Test	Result	Reference Range	Flag
ALT	27	7–35 U/L	
AST	23	10–35 U/L	

Thyroid

Test	Result	Reference Range	Flag
TSH	2.3	0.40–4.50 mIU/L	

Comment: On atorvastatin 40 mg.

Reviewed: Dr. Priya Raman, MD — April 22, 2026

Laboratory Report — September 1, 2026

Ordering Provider	Dr. Priya Raman, MD
Practice	Westchase Family Medicine
Collected	September 1, 2026 07:20 (fasting)
Performing Lab	Tampa Bay Clinical Laboratories, CLIA 10D0999999
Patient	Elena Ruiz | MRN GCV-012877

Complete Blood Count

Test	Result	Reference Range	Flag
WBC	6.3	4.0–10.5 K/uL	

Test	Result	Reference Range	Flag
Hemoglobin	13.7	11.5–15.5 g/dL	
Hematocrit	41.1	35–45 %	
MCV	89	80–100 fL	
Platelets	259	150–400 K/uL	

Basic Metabolic Panel

Test	Result	Reference Range	Flag
Sodium	138	135–145 mmol/L	
Potassium	4.3	3.5–5.1 mmol/L	
Chloride	102	98–107 mmol/L	
CO2	24	22–29 mmol/L	
BUN	15	7–20 mg/dL	
Creatinine	0.79	0.5–1.0 mg/dL	
Glucose	94	70–99 mg/dL	
Calcium	9.3	8.6–10.2 mg/dL	

Coagulation

Test	Result	Reference Range	Flag
PT	11.8	11.0–13.5 s	
INR	1.0	0.8–1.2	
aPTT	29	25–35 s	

Comment: Pre-procedure labs for left GSV ablation.

Reviewed: Dr. Priya Raman, MD — September 2, 2026

10. PLANNED PROCEDURE

Planned Procedure	Endovenous radiofrequency ablation, left great saphenous vein
CPT	36475 — Endovenous ablation therapy of incompetent vein, extremity, inclusive of all imaging guidance and monitoring, percutaneous, radiofrequency; first vein treated
Laterality	Left
Diagnosis	I83.812, I87.2
Surgeon	Dr. Elena Vasquez, MD, FACS
Facility	Gulf Coast Vascular Surgery Associates — office-based procedure suite
Payer	Florida Blue PPO
PA Status	Pending — Submitted September 23, 2026

Clinical Indication Summary:

• Left GSV reflux (SFJ reflux 2.1 s, GSV 7.2 mm at SFJ) on duplex 08/25/2026; no DVT, no deep reflux.

• CEAP clinical class C3: varicose veins with edema. Varicosities up to 5 mm on exam 04/14/2026 and 09/22/2026.

• Persistent pain, swelling, itching and cramping attributed to saphenous reflux, documented at every vascular visit 2023–2026.

• Symptoms significantly interfere with work (teaches seated, excused from standing duties) and daily activities (errands, walking).

• Graduated compression 20–30 mmHg worn continuously from 09/17/2024 through 09/22/2026 (24 months) with elevation, exercise and analgesics, without improvement. An earlier trial (02/13/2024 to early May 2024) was interrupted by the patient.

• History of contralateral provoked calf DVT (06/2025), treated and resolved; left deep system normal.

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